

CCIIO Enrollment Resolution and Reconciliation, Form 1095-A Data Reporting

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1. Introduction

1.1 Purpose

The Affordable Care Act (ACA) established new health insurance Exchanges to facilitate the purchase of health insurance products in the States and to offer financial assistance to qualified individuals (QIs) in the form of advanced premium tax credits (APTC) and cost-sharing reductions (CSR). The ACA requires integrated support to ensure individuals enrolled in Qualified Health Plans (QHPs) in the Exchanges are enrolled in the right QHP, at the right time, and with the right amount of APTC and CSR.

The Contractor performing work under this Statement of Work (SOW) will be responsible for identifying the root cause of enrollment and financial data discrepancies and resolving these discrepancies within the Federally-facilitated Exchange (FFE). For State-based Exchanges (SBEs), the Contractor will coordinate with each SBE to help resolve financial data discrepancies identified by issuers.

The Contractor will be focused on three primary work streams under this SOW:

- **Enrollment Resolution & Reconciliation (ER&R):** The purpose of the Enrollment Resolution & Reconciliation work is to provide the services necessary to reconcile discrepancies in enrollment and financial information shared between Health Insurance Exchanges (Exchanges), issuers of health and dental insurance, and the Centers for Medicare & Medicaid Services (CMS) in support of accurate enrollment data and financial payment data alignment.
- **Forms 1095-A Data Reconciliation and Reporting:** The purpose of the Forms 1095-A Data Reconciliation and Reporting requirement is to provide the services necessary to research and resolve data discrepancies reported by enrollees, issuers, the Internal Revenue Service (IRS), CMS, or any other allowable stakeholder regarding information that appears on FFE enrollees' Forms 1095-A.
- **Complex Case Help Center (CCHC):** The purpose of the CCHC requirement is to assist consumers with complex enrollment and eligibility issues related to their coverage through the FFE. These services are critical to improve the consumer experience, specifically for those consumers with complex eligibility cases or members of difficult-to-enroll populations. This support will help to enhance and improve enrollment and retention in the Exchange for those consumers with complex cases.

Data discrepancies for all three of these work streams will be provided for resolution and reconciliation. The Contractor will be expected to analyze all disputes and discrepancies, resolve all disputes, and communicate their disposition to CMS, issuers, consumers, Exchanges, and other appropriate stakeholders through automated and manual processes.

1.2 Background

1.2.1 Overview

Section 1311 of the ACA required each State to establish an Affordable Insurance Exchange beginning January 1, 2014 to provide individuals and small business employees with access to health insurance coverage. QIs are able to enroll in health coverage in QHPs as well as stand-alone dental plans (SADP). The Secretary of the Department of Health & Human Services (HHS) establishes and operates a FFE in any State that does not elect to operate a SBE. Within CMS, the Center for Consumer Information and Insurance Oversight (CCIIO) is the organization dedicated to ensuring successful implementation of, and adherence to, the specific provisions of the legislation related to private health insurance and coverage through the Exchanges.

The Federal Marketplace Programs System (FMPS) is the processing system established to operate, coordinate, and associate the Exchange programs, including enrollment processing. In order to ensure accurate enrollment information, CMS is required to reconcile enrollment records with all participating FFE issuers monthly, per 45 CFR part 155 Section 400(d). CMS has put processes in place for identifying and reconciling gaps between issuer and FFE enrollment records (referred to as “ER&R”, or enrollment resolution and reconciliation) and also for enrollment data collection between the SBEs and CMS for the purposes of payment.

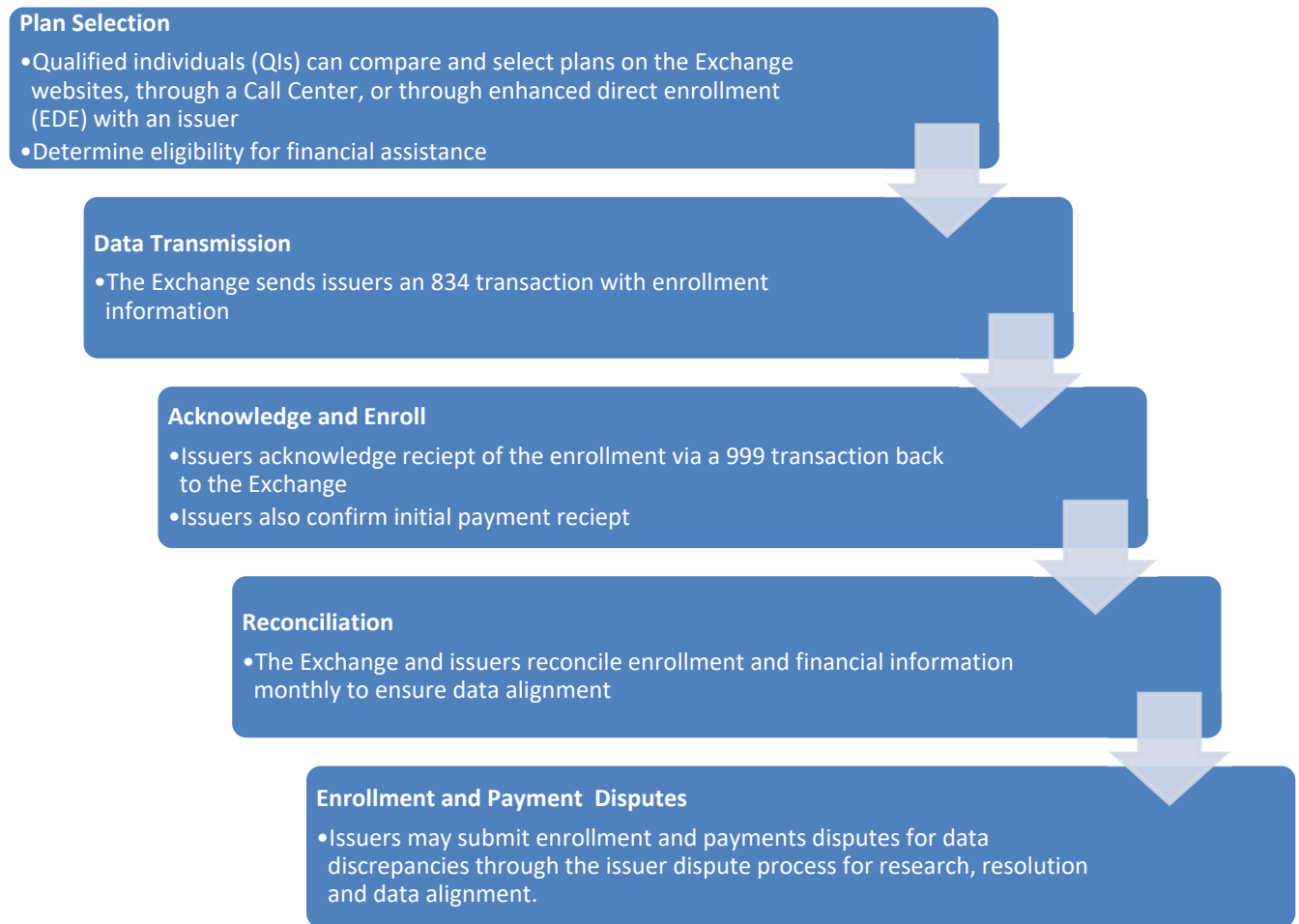
In addition, Section 1401(a) of the ACA requires that after the close of the tax year, the Individual Exchange will report to the Tax Filer and IRS on individual market policies purchased through the Exchange. The Exchange is required to file an annual report with the IRS and furnish the Health Insurance Marketplace statements, Form 1095-A, to individuals on or before January 31st each year.

1.2.2 Enrollment in Qualified Health Plans: Individual Overview

1.2.2.1 Individual Market

In most instances, QIs and existing enrollees may make selections for or changes to plan coverage during the annual scheduled open enrollment period and can qualify for Special Enrollment Period (SEP) changes throughout each plan year based on a qualifying life event. QIs may make selections for coverage in QHPs and SADPs through the FFE or SBE. These enrollments are processed by the applicable Exchange and the information is sent to issuers. Enrollees will be automatically re-enrolled in the coverage they had for the previous year if they do not actively select a new plan for the upcoming coverage year. Enrollment updates for these mid-year changes and SEPs must also be applied to the FFE and issuer data throughout the year.

Enrollees have multiple routes for making plan selections for coverage in the Exchanges. They can compare and select plans on the Exchange, contact the Exchange call center, or through the Enhanced Direct Enrollment (EDE) pathway. When enrollees make coverage selections, they can also provide information to determine their eligibility for financial assistance in the form of APTC and CSR. The Exchange must confirm the enrollment and exchange financial and enrollment information with the issuer. The issuers, in turn, must acknowledge receipt of the enrollment information and confirm when initial payment has been made to effectuate coverage.

Figure 1. Overview of Enrollment Processes

1.2.3 Enrollment Data Reconciliation & Dispute Processing

1.2.3.1 FFE

When an individual enrolls in coverage through the FFE or makes changes to their coverage, the FFE will send an enrollment transaction to the relevant issuer. In order to ensure the accuracy and completeness of the information transmitted and to maintain consistent information between issuers and the FFE, a process called enrollment data reconciliation is used.

The FFE and issuers will ensure the accuracy and completeness of enrollment and financial data through the enrollment data reconciliation process. At least monthly, issuers and the FFE will exchange enrollment and financial data files to verify the integrity of the enrollment transaction processing and resulting records. The FFE employs an automated monthly reconciliation process comparing certain data fields with issuer data.

First, the FFE sends issuers a “snapshot” of their current enrollment data in the form of a pre-audit file. Issuers use the pre-audit files to compare the information in their system with the FFE data and identify any missing enrollments.

Second, each issuer creates an inbound reconciliation file with their enrollment data. Issuers submit their files to CMS and an operational analytics contractor performs an automated data match between each issuer submitted file and the FFE enrollment data. The automated process matches records based on a unique collection of field information and resolves discrepancies through an automated set of business “rules” regarding discrepancy resolution. The operational analytics contractor creates an outbound reconciliation file for each issuer that describes the results of the reconciliation process and updates that need to be made by the issuer or the FFE. CMS makes its assigned enrollment reconciliation updates in the FFE. Issuers either make their assigned updates in their systems and reflect the change on the next inbound reconciliation file or submit a dispute to the ER&R Contractor.

Through the issuer dispute process, issuers may submit enrollment and payment disputes on data elements resolved through the automated process that they disagree with by sending disputes directly to the Contractor for research and resolution. Additional automated rules may be developed by the Contractor to process disputes. Any records that cannot be automated must also undergo a manual review process.

Finally, following the resolution of any ER&R discrepancies from issuer disputes, the Contractor then submits changes to the FFE or issuers to update their respective data.

1.2.3.2 SBEs

When an individual enrolls in a plan through a SBE, the SBE will send the enrollment transaction to the relevant issuer. At least monthly, the SBE will send an enrollment file that will be used for payments. CMS will use the file to make updates to internal enrollment records or will work with the SBE to resolve any discrepancies. SBE issuers are paid by CMS directly; therefore, SBE issuers may send disputes to the ER&R contractor for resolution in a manner similar to the FFE dispute process, but instead of updating the payment data, they send an informational report to SBEs and SBE issuers.

1.2.4 Form 1095-A (Health Insurance Marketplace Statement)

At the close of each year, based on the most complete and accurate FFE enrollment and financial payment data available for the coverage year through reconciliation, the Exchange files an annual report with the IRS and furnishes Forms 1095-A to individuals on or before January 31 to be used for their tax preparation.

If the information on an initial Form 1095-A is inaccurate, a corrected or void Form 1095-A must be issued to all parties. Form 1095-A stakeholders, their responsibilities, and information for which they are the authoritative source are outlined in the table below:

Table 1: Form 1095-A Stakeholders

Stakeholder	Responsibilities	Information for which Stakeholder is Authoritative Source
Individual Exchange	Provides data directly to the tax filer (or other relevant adult) and a copy to the IRS.	Individual-level information on individual market policies purchased through the Exchange.

Stakeholder	Responsibilities	Information for which Stakeholder is Authoritative Source
Tax Filer/ Other Relevant Adult	Receives individual-level information on individual market policies purchased through the Exchange.	
IRS	Receives a copy of data provided by the Exchange to the tax filer (or Other Relevant Adult). Accepts appropriate data transfers from the Exchange for tax administration.	Tax returns and supporting information filed with IRS.
CMS	Provides policy and guidance to the Exchange, coordinates the transmission of requests and responses among the Exchange and IRS through Data Services Hub, and records transaction history to support audits. Manages Data Services Hub.	Policy and guidance and verification transaction history.

1.3 Scope

The scope of this SOW consists of all labor, materials, and supporting services necessary to perform the task requirements to accomplish the efficient and effective reconciliation of enrollment and payment disputes, Form 1095-A consumer disputes and CCHC casework. The Contractor will be the central entity responsible for supporting Exchanges (FFE and SBE), interacting with CMS, issuers, and consumers with respect to all three work streams.

2. Assumptions

In performing functions under this SOW, the Contractor shall take the following assumptions and constraints related to all work streams into consideration:

- All references to days in the SOW reflect the business days of Monday through Friday and all support hours reference Eastern Standard Time (EST). Operations will not be required on Federal Holidays, unless deemed necessary by CMS.
- CMS estimates generated approximately 19.7 million initial Forms 1095-A in 2025 for plan year 2024 in electronic and printed format (volume is dependent on the number of active policies for the given coverage year). The FFE must transmit all consumer year-end tax related data to the IRS by the last day of January for the previous calendar year. CMS generates and transmits this data on a rolling basis starting in early January. CMS sends data to the IRS and consumers concurrently.

- In 2024, issuer dispute volume was approximately 2.2 million payment disputes, 500,000 enrollment disputes and 1.4 million SBE disputes. Approximately 99% of payment disputes, 95% of enrollment disputes and 100% of SBE disputes are resolved through automated processes and are considered low complexity with remaining percentage considered high complexity requiring a manual process because they could not be resolved through an automated process. Dispute volume in 2025 is estimated to increase by 10-20%.
- All consumers need to have accurate initial Forms 1095-A mailed January 31st each year. CMS estimates that up to 2% of consumers receiving a form will generate consumer disputes. There will be certain times of the year during which Form 1095-A consumer dispute volumes will vary, with the busiest periods occurring during and immediately after tax season (January-May). Individuals can qualify for tax extensions that will continue to impart a reduced volume of consumer disputes through October 15th. The Contractor shall be prepared to meet service level expectations throughout the tax season regardless of workload fluctuation.
- There will be certain times of the year during which activities will vary based on published regulations and CMS policies. The annual open enrollment period usually begins on November 1st and runs through December 15th. The specific start and end dates for the annual open enrollment period may fluctuate each year. CMS, at its discretion, can create special open enrollment periods. Consumers who purchase QHPs through the FFE or SBEs can enroll or modify their coverage during the year with a qualifying life event. Enrollment resolution and reconciliation volumes will peak during and up to 120 days after the close of the annual open enrollment period.
- CMS estimates approximately 24 million individuals have enrolled in the Marketplaces in 2025.
- The Exchanges will be operational for data exchange except for scheduled outages. Any software used by the Contractor will have to be compatible with the CMS Data Service Hub and CMS Desktop requirements for purposes of compatibility for all deliverables. Any solution must have the capability to seamlessly interface with the CMS Data Services Hub using a Simple Object Access Protocol (SOAP) message format and a Hypertext Transfer Protocol Secure (HTTPS) transport protocol. The Exchanges observe software integration lock out periods during the open enrollment period which usually starts 15-30 days before Open Enrollment and continues through the end of Open Enrollment. The Contractor shall plan and prioritize testing and deployments accordingly to avoid delays in implementing releases to meet business needs.
- The Contractor shall ensure compliance with all CMS security protocols, including implementing any procedures to protect security and Personally Identifiable Information (PII) data.
- The Contractor shall conduct internal audits to ensure compliance with CMS guidance, including but not limited to Technical Review Architecture and Security standards requirements of this SOW, and for the prevention of fraud, waste, and abuse.
- The Contractor shall not design, implement, or maintain any customized or proprietary utilities or software without prior approval from CMS.

- All deliverables shall be compliant with Section 508 requirements of the Rehabilitation Act.
- Virtual Classes: All virtual classes must be delivered using a 508-compliant platform and shall be compliant with Section 508 of the Rehabilitation Act.
- Course Materials: All course materials (hard copy or electronic presentations, handouts, aides, and guides) shall be compliant with Section 508 of the Rehabilitation Act and delivered to all participants at least five (5) business days before the start of class. However, the parties understand that this time period may be shortened due to unanticipated circumstances, for example, ad-hoc trainings, trainings to address urgent matters, etc. An electronic version (i.e., electronic PDF or other types of electronic files) of the training materials should be made available to CMS.

Performance under this SOW will require a significant level of coordination and collaboration with CMS, other government agencies (including federal agencies and States), and other CMS contractors. Coordination with the CMS Data Services Hub, Eligibility Support, Federal Exchange, Marketplace System Integrator, Operations Analytics, Operations Support, and the Stakeholder Engagement contractors is expected. The Contractor shall alert CMS to problems that may involve areas of other contractors' responsibility. The general responsibilities of those contractor support functions are described below:

CMS Data Services Hub (DSH) Contract:

The CMS Data Services Hub (DSH) contractor provides support to CMS in a range of various Hub operations to provide centralized and standardized information technology systems across the Exchange and federal and state health programs and various Federal Government agencies. In this capacity, they may support transaction systems operating between the Federal and State Exchanges, Federal Government agencies, and QHP issuers.

Eligibility Support Contract:

The Eligibility Support contractor will assist with the intake, routing, review, and troubleshooting of applications, changes, and redeterminations submitted for enrollment into a QHP and for insurance affordability programs including, but not limited to APTC, CSR, Medicaid, Children's Health Insurance Program (CHIP), and the Basic Health Program (where applicable). Changes in eligibility and issue resolution by that contractor could generate issues and workload for the ER&R and Form 1095-A Contractor (and vice versa).

Federal Exchange Call Center Contractor:

The Federal Exchange Call Center contractor is responsible for receiving inbound consumer calls 24x7, respond directly to calls when appropriate, transfer calls to the appropriate area, or escalate calls to CMS. The Federal Exchange Call Center contractor can provide Tier 1 support to consumers to address Form 1095-A related issues.

Federal Exchange Print Contractor:

The Federal Exchange Print contractor is responsible for the receipt of notice files to be printed (EFT), and the execution of printing and mailing files within the agreed upon timeframe. The Federal Exchange Print contractor also provides error reports and advanced

metrics in addition to ad-hoc support with regards to technical requirements needed to comply with their semi-automated processes.

Federal Marketplace Program Systems (FMPS) Contract:

The Federal Marketplace Program Systems (FMPS) contractor is working with CMS to develop lifecycle management of web applications supporting a range of functional areas including Eligibility & Enrollment, Plan Management, Financial Management, Oversight, Communications, and Customer Service. The FMPS Contractor will also be responsible for performing nationwide end-to-end integration testing to validate the effectiveness of the nationwide FMPS.

Marketplace System Integrator (MSI):

The Marketplace System Integrator (MSI) contractor is responsible for managing, synchronizing and coordinating the integration of all Marketplace systems, system interfaces, and end user interfaces.

Operations Analytics Contract:

The Operations Analytics contractor will provide analyses of operational information that CMS will need to monitor and evaluate various program functions across various programs operating under the Exchange, including the exchange and reconciliation of data related to enrollments. Enrollee data must be kept in sync between the Subject Matter Experts (SMEs), FMPS, and issuers when a State operates an Exchange, and between the FFE and issuers in the case of an FFE. This contractor is responsible for the monthly automated data match and reconciliation process between the FFE and issuers. This includes generation of the Pre-Audit File, the reconciliation outbound file (RCNO), and FFE Update Files. The Operations Analytics contractor will contribute towards the development of deficiency reports to be forwarded to ER&R and Form 1095-A Contractor from dispute updates produced for the FFE Update Files.

Operations Support Contract:

The Operations Support contractor will provide Tier 2 and Tier 3 specialized program support and work in conjunction with other CMS contractors and CMS staff for inquiry resolution. The core areas of operations support for eligibility and enrollment operations include support to analyze HIPAA standard ASC X12N 834 Benefit Enrollment and Maintenance transaction (834) errors.

Stakeholder Engagement Contract:

The purpose of the Stakeholder Engagement contract is to develop and implement a variety of training programs for the issuers and other entities that support the issuers through the Exchanges (agents and brokers) as well as the SBEs. The Stakeholder Engagement contractor is responsible for all aspects of the training program including development of the materials that will be used in each type of training program. The materials may be newsletters, guidance documents, FAQs, user manuals, operational documentation etc. The Stakeholder Engagement contractor will also be responsible for all logistics of training.

3. Description of Requirements

The Contractor shall perform the following tasks and deliver requirements in accordance with stated performance standards:

- The Contractor shall perform all work described in this SOW consistent with the stated, scope and assumptions. In performance of the tasks, the Contractor shall assign personnel with relevant knowledge and experience to the tasks described. Since workloads will fluctuate considerably during the period of performance, the Contractor's staffing processes must be flexible and capable of adequately responding to variations in the volume and workload. Where feasible, the Contractor shall cross-train employees to minimize redundancies and to ensure a balanced workforce.
- The Contractor shall ensure compliance with laws and regulations related to Personally Identifiable Information (PII) Protected Health Information (PHI) and Federal Tax Information (FTI); and understand and implement relevant CMS approved standards and practices regarding CMS technical and physical security. The Contractor shall comply with standards endorsed or adopted by the Secretary of HHS pursuant to Sections 1104 and 1561 of the Affordable Care Act, Health Insurance Portability and Accountability Act (HIPAA) transaction standards, including accessibility, security, and privacy standards consistent with Federal law.
- The Contractor shall ensure that all tasks are completed in a timely and accurate manner consistent with approved Standard Operating Procedures (SOPs). As such, the Contractor shall maintain a detailed awareness of all aspects of its performance on the tasks, to include activity status against schedules and planned milestones and cost status.
- The Contractor shall assign personnel with relevant experience and knowledge in enrollment resolution and reconciliation processes including, as applicable, an in-depth knowledge of the Accredited Standards Committee (ASC) X12 integration and operations of systems that support enrollment reconciliations, associated Information Technology (IT) infrastructure; ASC X12 834 transaction processes and mandated electronic enrollment standards currently in use by issuers and employers; and knowledge of 820 payment file functions related to enrollment resolution and reconciliation issues.
- The Contractor shall follow any CMS-established SOPs, including any CMS-approved Contractor developed SOPs in performance of the tasks in this SOW.

The specific SOW task requirements are further described below. Unless otherwise specified, the Contractor shall perform the task requirements described in this SOW in the base period and any option period exercised under the task order.

3.1 Task 1: Program Implementation and Oversight

The Contractor shall plan, monitor, and control all tasks under this SOW. The Contractor shall follow an accepted program implementation methodology to effectively perform the following:

1. Project Implementation and Oversight
2. Financial Planning and Management
3. Risk Planning and Mitigation
4. Issue Resolution
5. Communications with all stakeholders, including CMS

Kickoff Meeting and Project Implementation and Oversight Plan (PIOP)

The Contractor shall participate in an initial contract kickoff meeting hosted by CMS to discuss expectations for all tasks under this task order. Other CMS stakeholders may be invited to attend at the COR's discretion. In consultation with the COR, the Contractor shall prepare decision artifacts and milestone updates, including an agenda and handouts (i.e. decision artifacts and milestone updates) for the meeting as determined in consultation with the COR. The Contractor shall present its Project Implementation and Oversight Plan (PIOP) and overall approach for meeting CMS' requirements. The Contractor shall also present its Implementation and Oversight Team and Key Personnel during the contract kickoff meeting. The contract kickoff meeting shall occur no more than twelve (12) business days after the start of the base period, unless otherwise determined by the COR.

Within seven (7) business days of the start of the base period, the Contractor shall deliver a draft PIOP that lays out the Contractor's approach, timelines, detailed schedule and milestones, and tools to be used in execution of the task order unless otherwise determined by the COR. The PIOP shall describe, in detail, the approach to be used for each aspect of the tasks. In addition, including risk management and technical management approaches, work breakdown structure (WBS), schedule management approach, schedule, and key personnel.

The PIOP shall also delineate contingencies, resourcing and alternate work strategies, in response to unexpected challenges that may arise. The PIOP shall describe the management of staff, the documentation review process, and processes for resolving complex enrollment resolution and reconciliation issues. The PIOP shall also include how the Contractor plans to set up, test, and cutover any information systems and interfaces, and coordinate reporting requests as identified within the SOW.

As part of the PIOP, the document shall outline the process supporting technology and AI transparency in an organized fashion, including documenting where each technical solution is housed and its purpose. Additionally, the PIOP shall outline a proposed structure and collaboration process with CMS staff and other key stakeholders.

The Contractor shall update and maintain the CMS-approved PIOP throughout the period of performance.

Within two weeks following the kickoff meeting, the Contractor shall submit an updated PIOP that addresses all project management issues and decisions addressed during the kickoff meeting, including any necessary changes to timelines. The final PIOP shall be due fourteen (14) days after the Contractor receives CMS' comments on the draft project design report or 60 days after the start of the base period.

COR approval will be required before any major staffing or ODC commitments are made in performance of the tasks.

Reporting Requirements

All reports shall be submitted in an electronic format compatible with CMS software applications. If requested by the COR, the Contractor shall provide hard copy reports and draft report templates and outlines for approval. The Contractor shall comply with all reporting requirements for documents released to the public, as described in the task order.

Monthly Progress, Data and Cost Report

The monthly progress, data and cost report is due by the third Monday of every month, two days prior to the Monthly Budget Review meeting or as agreed upon with the COR. CMS will provide a template for this report after award of the contract. The report shall include a narrative of the prior month's activities by task and activity, such as:

1. A summary of task activities.
2. Summaries of meetings and actions for the upcoming month.
3. Outstanding issues that require CMS action.
4. Any unresolved issues from the prior month.

Additional reporting elements may be required in this report and will be directed by the COR as necessary. The report shall also indicate, at a minimum, the current monthly costs, cumulative to-date costs, and remaining funding. The report shall track costs by:

1. Labor categories
2. Hours per labor category by task and, where applicable, sub-task
3. Labor rates
4. Subcontractor costs
5. Travel costs (as applicable)
6. Material, Fees, and Other Direct Costs (ODCs)
7. Subtotal direct costs
8. Indirect costs
9. Total costs, in aggregate and by task and, where applicable, sub-task
10. In any other format as directed by the COR

Form 1095-A Consumer Disputes and Enrollment Process Improvement Report

The Contractor shall prepare and submit a monthly report that identifies a prioritized list of proposed improvements or recommended changes to the Form 1095-A consumer dispute and enrollment processes including discussion of the rationale for each proposed change. The report must address recommendations made under other tasks and their level of adoption by CMS. Additionally, the report shall include, but is not limited to the following:

1. Situations suggesting the need for increased education of FFE, QHP, SBEs, and contractor staff or other stakeholders
2. Recommendations for changes to the Form 1095-A triage or enrollment processes to reduce the number of open cases
3. Recommendations for additional system integration and data inconsistencies
4. Recommendation for changes to supporting information systems to improve efficiency

5. Identification of changes to regulations or guidance necessary to authorize improvements
6. Identification of the benefits and costs associated with implementing changes
7. Ways to improve communication and coordination between stakeholders, contractors, FFEs, QHPs and SBEs
8. Recommendations to decrease the amount of data churn between Enrollment Data Alignment processes

Task Order Summary Report

At the conclusion of the task order, the Contractor shall issue a summary report to CMS describing the work completed during the task order and recommendations to improve the review processes. The Contractor shall submit a draft version of the report in the 10th month of the final period of performance. The Contractor shall revise the report according to the COR's direction and submit a final/revised report 30 days before the end of the task order. The final/revised report shall be submitted to the COR and the CO. This report will occur only once during the life of the contract.

Weekly Meetings

The Contractor shall participate in weekly meetings with the COR and other CMS stakeholders beginning the week after the initial kickoff meeting. The purpose of these meetings is to review progress, identify risks and issues, and discuss mitigation strategies. At least one day prior to the meeting, the Contractor shall send a proposed agenda highlighting, by task, key activities any risks or issues encountered as directed by the COR. Within two business days following the meeting, the Contractor shall provide minutes of such meetings documenting any decisions made or plans developed. These meetings will be at a set time and will be of sufficient duration to provide a comprehensive update of project activities and discuss any program management and administrative issues. The COR may cancel or modify the format and agenda of individual weekly meetings as determined necessary. It is anticipated that less formal communications will also occur on a more frequent basis, as needed, between the COR or other assigned CMS staff and the Contractor. However, only the COR may provide technical direction to the Contractor.

Periodic In-Person Meetings with CMS

As requested by CMS, the Contractor shall meet with CMS staff in the Baltimore, MD, or Bethesda, MD/Washington, DC metropolitan area, another CMS Regional Office, and/or onsite at the Contractor's facility at designated intervals during the course of the project. The purpose of the meetings is to review work progress, performance evaluation and quality improvement activities, and future plans under the task order. The Contractor shall plan for at least four in-person meetings per year and shall send a proposed agenda at least one day prior to the in-person meetings and submit minutes of meetings within two business days or at a time period as determined by the COR. Any in-person meeting can be cancelled, rescheduled, or changed to a virtual format at the discretion of the COR.

3.2 Task 2: ER&R Issuer Disputes

The Contractor shall perform the core functions of processing, reviewing, and resolving data discrepancies. The Contractor is responsible for developing and executing resolution and reconciliation activities, as well as designing solutions for resolving operational and data issues; establishing and documenting internal controls, and oversight to address emerging issues while informing CCIIO in a timely manner. The Contractor is responsible for updating enrollment records; reconciling issues related to payments with issuers and FFE or SBEs; electronic file transfers; manual overrides, and the ability to perform batched record corrections affecting few or numerous records across one or more QHP or FFE States.

The issuer dispute process permits FFE issuers to submit enrollment disputes using the Enrollment Dispute Form via the Electronic File Transfer (EFT) Server or System of Exchange Enrollment Data (SEED). The Contractor shall use automated processes to accept files and notify the issuers of file receipt via email.

There are multiple types of disputes that will be accepted such as:

- Standard dispute—items where the issuer does not agree with the disposition flag or record-level flag assigned by Analytics Contractor through the monthly automated reconciliation process
- Enrollment disputes—applicable when the issuer does not agree with data within an enrollment policy or record in the FFE
- Auto-Renewal disputes—applicable when a passive reenrollment has been sent in Error and the issuer needs it removed from the FFE
- Reinstatements—applicable when the issuer needs to reinstate members' policies for coverage
- Health Insurance Casework System (HICS) direct disputes—applicable when issuers assign HICS cases directly to the ER&R Contractor for certain dispute types
- Enrollment Blocker disputes—applicable when a consumer with an existing enrollment on the FFE successfully updates their current application through the Exchange but a technical issue prevents the policy from updating with the new eligibility information
- (SEED) direct disputes—applicable for issuer disputes submitted through SEED
- Post Deadline Inaccuracy (PDI) disputes – applicable for issuers that identify data inaccuracies that impact payment after the regulatory deadline

The Contractor shall ensure the effective resolution and reconciliation of enrollment data discrepancies related to payments and other data elements, using the Policy Payment Report (PPR) or monthly HIX 820 payment transactions from CMS to issuers as a reference point. FFE issuers submit payment discrepancies through the PPR-820 Dispute Form to initiate the Payment Dispute Process. SBE issuers submit payment disputes through the SBE Payment Dispute Form.

Issuers submit disputes to CMS via EFT or SEED using the PPR-820 Dispute Form or the SBE Payment Dispute Form. The Contractor evaluates, investigates, and responds to Payment Disputes. Initially, the issuer receives the PPR-820 Dispute Response File with an initial dispute status and a CMS tracking number. Additionally, issuers receive the Semi-Monthly Detailed

Report with up-to-date statuses for all disputes.

Issuer disputes shall be categorized as either low or high complexity based on whether there is an automated dispute rule or whether it requires manual review and resolution. See sections 3.2.1 and 3.2.2 for details. For all disputes:

- The Contractor shall receive the incoming electronic data errors/issues transactions for the FFE through an issuer dispute process. Data resolutions and other changes will be transmitted to the Operations Analytics Contractor with updates for existing enrollees, new enrollments, terminations, changes, and corrections. All updates should occur through electronic data transactions.
- The Contractor may receive electronic and/or paper documentation as determined by CMS regarding enrollment errors and will use such information to resolve issues related to enrollments and other error related issues associated with the electronic data interface. The Contractor shall follow and ensure appropriate documentation practices in accordance with SOPs. The Contractor shall ensure cross-coordination with other CMS systems and operations in the handling of any electronic data interface transactions that did not pass specific system edits for the file to be successfully transmitted.
- The Contractor shall follow a process for reviewing disputes for specific data elements. The Contractor shall identify the correct sources of data or information needed to resolve disputes including FFE extracts, 834 transactions, pre-audit data, reconciliation files, HICS information, and issuer information. Additional sources of information may be used as necessary to determine the correct information to be updated in the FFE, issuers or both.
- The Contractor shall research, recommend, and develop improved approaches and best practices for enrollment processing and reconciliation functions, including but not limited to retroactive enrollments, terminations, enrollment changes; reinstatements, status adjustments, and membership maintenance updates submitted by the FFE and providing CMS with recommendations to enhance efficiencies.

Table 3: Service Level Agreement – Issuer Disputes

Issuer Disputes	Description	Target Resolution Time	Resolved Status
High Complexity	Manual issuer disputes generated from a dispute form	95% within 10 days	Initial resolution date

3.2.1 Subtask 2.1: Low Complexity Issuer Disputes

The Contractor shall resolve and process low complexity issuer disputes. Low complexity issuer disputes are defined as disputes resolved through the automated processing of rules that apply to the different types of disputes.

The Contractor shall provide CMS with analytics on the types of disputes submitted by issuers by type of dispute and by issuer. CMS and the Contractor may use these analytics to improve the dispute resolution process.

The Contractor shall work with CMS on the development and modification of business rules for processing issuer disputes including the ongoing quality assurance of current rules. Business rules provide logic for completing automated and manual dispute resolution. Automated processes should be used to achieve dispute resolution whenever possible. To develop new rules, modify existing rules, and validate current rules, the Contractor shall analyze disputes.

3.2.2 Subtask 2.2: High Complexity Issuer Disputes

The Contractor shall resolve and process high complexity issuer disputes which are cases that require a manual process because they could not be resolved through an automated process. The Contractor shall support the resolution of disputes by utilizing and enhancing various pathways such as APIs and the Issuer and Enrollment System (IES).

The Contractor shall review and resolve record matching issues from the automated reconciliation process. These include records reported by the issuer that may not match on the FFE side (Unmatched “I” records, UIRs). The Contractor shall process “I” records using automation to identify a possible match. The UIR system designates a category for these records. Each category provides an associated definition and guidance for the issuer to resolve the issue. Records not categorized by the automated process are manually reviewed and assigned a category and sent to issuers in a monthly UIR report. The UIR process also includes manually generating Forms 1095-A and manual payments adjustments for UIR categories as defined by CMS.

The ER&R contractor processes SBE and FFE Post Deadline Inaccuracy (PDI) disputes, handling payment discrepancies identified after regulatory deadlines. These disputes must be compared to audit findings and addressed through a manual payment adjustment rather than through automated enrollment updates. Key functions include processing dispute forms, generating monthly summary reports, conducting biannual batch processing, preparing and distributing attestation forms via electronic file transfer, and generating final manual adjustment files for payment approval.

The Contractor shall represent disputes work during technical discussions with CMS and the enrollment data alignment teams regarding broader updates to enrollment data alignment, such as daily operational reconciliation technical workgroup calls.

The Contractors shall update dispute operational documentation and review related enrollment data alignment documentation to provide technical assistance and feedback regarding their work.

The Contractor shall provide CMS with monthly analytics on the types of disputes submitted by issuers categorized by type of dispute, issuer, when received and aging reports. CMS and the Contractor may use these analytics to improve the dispute resolution process.

The Contractor shall perform advanced data analytics across all enrollment data alignment work streams (Form 1095-A consumer disputes, issuer disputes, enrollment reconciliation and 834) in order to make recommendations to CMS on enrollment data alignment improvements.

The Contractor shall provide three to four one on one technical assistance calls with issuers per month to resolve recurring and outstanding dispute data issues.

The Contractor shall use the JIRA process already established to manage, track and resolve ER&R dispute business rules, bug fixes, change requests, research and analytics requests.

The Contractor shall manage and post ER&R documentation such as reports, minutes, presentations and business rules on Confluence.

The Contractor shall support all CMS weekly issuer calls and SBE calls with states on a monthly basis. In addition to preparing 508-compliant issuer educational materials, slide presentations, and technical documentation.

The Contractor shall provide support for manual payment processes associated to SBE and FFE disputes.

The Contractor shall provide analyst support for the monthly Enrollment Alignment Performance Summary (EAPS) reporting and Account Manager Dashboards including training. The Contractor shall analyze new Marketplace enrollments for data anomalies consistent with unauthorized plan switches (UPS) and unauthorized enrollments (UEs). Anomalies may include patterns associated with accidental or intentional obfuscation of demographic data which bypasses overlapping coverage rules, leading to undetected overlaps. The Contractor will recommend and implement mitigations to include cancellations of undetected overlaps, enhancements to overlap detection logic, and reporting of these enrollments to CPI. The Contractor will monitor data trends and recommend logic enhancements to quickly address new patterns and scenarios as they arise. The Contractor shall provide support to CMS stakeholder-driven initiatives including those proposed by the Office of Program Operations & Local Engagement (OPOLE) and the Center for Program Integrity (CPI) to enhance casework processes, as approved by the COR and/or CMS Business Owners. This support will involve collaboration to identify opportunities for data alignment, overlap identification across ACA and Medicare, and the recommendation of effective solutions and process optimization such as rules-based model; to ensure the development and implementation of improved casework processes that streamline workflows and enhance efficiency.

3.3 Task 3: Form 1095-A Consumer Disputes

The Contractor shall perform the core functions of processing and researching all consumer disputes related to Form 1095-A discrepancies within the timelines agreed upon between CMS and the Contractor. Core functions include monitoring caseload and financial data transactions and processing, reviewing and resolving data discrepancies. Core functions also include developing and executing resolution and reconciliation activities and designing solutions for resolving operational and data issues. The Contractor is responsible for establishing and documenting internal controls and oversight to address emerging issues and informing CMS in a timely manner, maintaining Form 1095-A discrepancy resolution transactions, updating enrollment records when appropriate, reconciling issues related to APTC payments with issuers. The Contractor is responsible for researching and resolving consumer issues for IRS Form 1095-

A, electronic file transfers, manual overrides, and the ability to perform batched record corrections affecting few or numerous records across one or more QHPs or FFE States. The Contractor is responsible for collaborating internally among teams supporting other Project Tasks, including teams supporting ER&R issuer disputes, EDA CORE, as well as with any applicable external stakeholders as part of implementation of the core functions.

Processing Incoming Consumer Disputes, Errors/Issues Transactions

The Contractor shall download cases daily from HICS for all Form 1095-A consumer disputes and the source file information into the CCIIO Enrollment Resolution and Reconciliation System (CERRS) described in section 8.1 to support analysis of the HICS cases and reporting.

To expedite and streamline processing of Form 1095-A and tax-specific issue resolution of enrollment and/or other activities, the Contractor shall continue to automate the resolution and reconciliation and intake processes as much as possible. The Contractor shall ensure that its processes include appropriate audit trail mechanisms (e.g., time/date stamping) so that actions taken can be tracked, reviewed and reported in a timely manner and prevent enrollment cases from aging beyond established timeframes. The Contractor shall explore ways to utilize automation including development and use of AI tools for timely and accurate resolution of Form 1095-A consumer disputes.

The Contractor will categorize the Form 1095-A consumer disputes into low, medium, and high complexity based on case type. Consumer Dispute case types are determined based either on the issue articulated in the HICS case narrative or the nature of the process that needs to be followed. The automated process that loads CERRS performs the initial case type assignment analysis. Manual review is required when the automated process is unable to assign a case type. Details regarding each category can be found in sections 3.3.1, 3.3.2, and 3.3.3.

The Contractor shall follow and ensure appropriate documentation practices in accordance with SOPs. The Contractor shall ensure cross-coordination with other CMS systems and operations in the handling of any electronic data interface transactions.

Identifying and Communicating Trends

In accordance with SOPs or CMS guidance, it is the Contractor's responsibility to identify trends as they develop, with regards to incoming Form 1095-A consumer disputes. The Contractor shall develop creative solutions to best address the larger trends and the trends with most consumer impact more efficiently and effectively, including, but not limited to, processing correlated cases in a streamlined way.

Communicating trends that have been identified to all stakeholders in a timely fashion shall be a priority. Often the consumer dispute queue is the end of the enrollment pipeline and, with guidance from the Contractor, CMS can reduce or eliminate inappropriate and simplistic consumer disputes with education and outreach. Trends also help CMS identify priorities which will drive the focus of the work in subsequent tasks. The Contractor shall collaborate internally with the team working on ER&R issuer disputes to help identify trends across issuer and consumer disputes.

The Contractor shall research and resolve escalated issues in data errors related to IRS Form 1095-A using Exchange systems. The Contractor will contact consumers via outbound calls and

correspondence (mailed by separate print contractor), as appropriate, to clarify issues, complete research, and inform consumers and other parties about issue resolution.

Disseminate Consumer Disputes for Research and Resolution/Processing Retroactive Actions

After incoming consumer disputes have been identified, it is the Contractor's responsibility to assign cases to their appropriately-trained staff to be worked. Case assignments should be based on the experience level of employees and case complexity.

CMS encourages any employee specialization and automation that introduces efficiencies into the research and resolution process. CMS may also request that specific teams be created to focus on unique issues to be outlined in the SOPs or presented on an ad-hoc basis.

CMS may request the Contractor to prioritize certain consumer disputes. The Contractor shall provide a point of contact for high-priority resolution also known as "Form 1095-A VIP cases" and provide a mechanism to prioritize cases as necessary.

The Contractor shall perform retroactive enrollment adjustment functions for enrollments, terminations, and changes, for the FFE and provide informational and analytical reports to CMS. It will also identify areas requiring retroactive adjustments for SBEs, for which the SBE must make the actual changes.

Upon identifying or receiving information about discrepancies, the Contractor may be required to take retroactive action. In such instances, the Contractor shall:

- Record the requested adjustments
- Validate any documentation that the Exchanges and QHPs submitted to ensure they fall within the Exchange Guidance prior to processing the request
- Research identified issues and take corrective actions to resolve
- Generate and submit reports for CMS review on any discrepancies or issues determined necessary by the COR
- Utilize the FMPS for distributing the Final Disposition and report to all FFE issuers on such retroactive issues. The report shall contain detailed actions taken and corrective actions if known for each request.
- The Contractor shall follow the policies and procedures that are in place at CMS for the completion of all retroactive adjustments and shall notify the COR whenever the inventory of work is expected to exceed service levels established in the task order.

The Contractor shall ensure the special issues are resolved in a timely manner as described in the Key Performance Indicator portion of section 3.9 in this SOW. The Contractor shall provide status and final resolution updates either electronically through the Data Service Hub or through CMS to the inquirers for operations or processes that are outside of the normal workflow.

The Contractor shall document status and resolution and keep track of all the information that is required for the resolution of the issues in the Data Services Hub and/or FMPS or other system identified by CMS. The Contractor shall provide monthly reports, issue logs, an action plan to address the issues, and a target completion date to close out each issue. Issues related to APTC, and advance CSR amounts shall be reported in distinct portions of the reports, issue logs, and action plans to facilitate communication with CCIIO's Payment Policy and Financial

Management Group (PPFMG). The COR will determine if the Contractor's proposed date is acceptable and provide alternative directions if necessary.

Consumer dispute research will conclude with a final determination driven by CMS approved decision rules or by conclusions drawn from casework assistance calls with CMS. Once a final determination is made, it is the responsibility of the Contractor to submit changes to the FFE that will correct the error (if identified), modify the status of the casework in both the case management system and any applicable CMS systems.

If more information is required before a final determination can be made on a complex case, it will be the responsibility of the Contractor to complete issuer, consumer, or CMS Office of Program Operations & Local Engagement (OPOLE) Caseworker outreach. As with final determinations, these actions must be tracked via case status updates. Upon receiving the information necessary to close the case, it is expected that the final determination be made promptly, as defined by the timelines represented in the SOPs.

At the request of CMS, the Contractor must be able to quantify these case status changes on a regular basis.

Service Level Agreement – Form 1095-A

The Contractor shall issue a final determination on each case in accordance with the following Service Level Agreement (SLA):

Table 4: Service Level Agreement – Form 1095-A

Form 1095-A Case Type	Description	Target Resolution Time	Resolved Status***
Low	Mailing Address Updates (MAUs) and other automated dispute types	2 business days	Resolved after the change is submitted in queue for processing
Medium	Multiple data sources required for validation; issuer and/or consumer outreach may be needed for validation	5 business days*	Resolved after consumer is notified of the case resolution and change, if needed, is submitted into queue for processing
High	Multiple data issues and multiple data sources required for validation and may require additional outreach needed for resolution. Case types include VIP, CMS-directed, Multiple and Total Monthly APTC	10 business days**	Resolved after consumer is notified of the case resolution and change, if needed, is submitted into queue for processing

*CMS will specify business rules to address scenarios in which an external source is unresponsive and prevents closure of a case.

**CMS will provide specific guidance to the Contractor on a case-by-case basis for high complexity cases involving CMS input in which 10 business days is unrealistic to provide a final determination.

***Case resolution times referenced in SLA will not be impacted by delays to the FFE update schedule outside of the Contractor's control. The Contractor will be expected to resolve all cases in each category received within the set number of days of the next update run. For example, the Contractor may be expected to resolve reprint requests received up to one business day before a weekly reprint run. Cases received inside the SLA window may be processed for the next run.

Cases that require escalation to CMS, another entity, or that are correlated and pending resolution by the issuer shall be excluded from the low, medium, and high resolution timeframe expectation.

The Contractor shall plan to recruit, onboard, and train approximately 100-120 temporary case analysts and 4 temporary team leadership roles to handle the increase in Form 1095-A consumer disputes received during the tax season (January-May) and off board temporary cases analysts and team leads after the tax season. Case volumes may triple during the tax season. The number of temporary employees may vary each year depending on projected case volume and other business needs. The Contractor shall develop and utilize an efficient process for onboarding and managing temporary employees to meet business needs in conjunction with CMS while adhering to all HHS and CMS security requirements.

The Contractor shall provide the necessary support to address special issues that arise from enrollment processing areas from Exchange entities including:

- Transaction Data Acceptance and Rejection of Data Files.
- Enrollment resolution and reconciliation of Individual Enrollment.
- Plan Comparison, Selection and Enrollment.

Special issues are defined as any enrollment issue or reconciliation activity that cannot be completed or resolved within a standard processing time without CMS-determined resolution steps and guidance. These issues may emerge from any of the enrollment transactions previously identified, policy and procedure changes, public relations inquiries, and Congressional inquiries including manual efforts to complete the enrollment process. The Contractor shall identify, research, analyze, and manage all incoming and outgoing electronic data transactions and correspondence, if applicable, related to these issues. Such analysis includes identifying the root cause of the issue, the frequency of the issue over time, development of recommended approaches for resolution and deterrents to prevent future occurrences, where possible. The Contractor will coordinate discrepancy resolutions with SBEs, FFEs, State Agencies, the Eligibility Support Contractor, or issuers as is appropriate and necessary to achieve a final, coordinated resolution.

3.3.1 Subtask 3.1: Low Complexity 1095-A Disputes

The Contractor shall process and resolve Form 1095-A low complexity consumer disputes (Case type: mailing address update) within two business days. The case is considered resolved after the change is submitted in queue for processing.

3.3.2 Subtask 3.2: Medium Complexity 1095-A Disputes

The Contractor shall process and resolve Form 1095-A consumer disputes requiring multiple data sources for validation as medium complexity cases within five business days. Special Enrollment Period (SEP) mailing address corrections shall be included as a medium case type. The case is considered resolved after the consumer has been notified of case resolution and the change, if any, is submitted into the queue for processing.

3.3.3 Subtask 3.3: High Complexity 1095-A Disputes

The Contractor shall process and resolve Form 1095-A consumer disputes that are received or become high complexity based on one of the following criteria:

- Case type VIP: Based on HICS issue flags (SWIFT, Media Interest, Congressional, Elected Official, Level 1).
- CMS directed: In scenarios where CMS directs the contractor to escalate a case to VIP status, the contractor will change the case type to VIP and update the Level to “1” in HICS.
- Case type Multiple: Cases which have multiple data issues requiring multiple data sources for validation and may require additional consumer outreach for resolution.
- Case type Total Monthly APTC: Cases which have an APTC discrepancy requiring multiple data sources for validation and may require additional consumer outreach for resolution.

Cases that are deemed “high-complexity” should be resolved within ten business days. The case is considered “resolved” after the consumer is notified of case resolution and change, if needed, is submitted into the queue for processing.

3.4 Task 4: Complex Case Help Center (CCHC)

The Contractor shall investigate and assist in resolving complex and/or high-profile cases. These cases may occur when an issuer is unable to determine the appropriate course of action to assist a consumer, resulting in the need for additional guidance or intervention. The Contractor shall provide:

- Advanced casework support and resolution for complex consumer cases
- Specific instructions to consumers, assisters, agents/brokers, and/or issuers regarding actions necessary to resolve cases

The Contractor shall focus on identifying consumers referred from In-Person Assisters, including but not limited to Navigators and/or Certified Application Counselors, with complex case issues, or those flagged through other mechanisms as having complex case issues, who are seeking to obtain or maintain coverage through the FFE and are in need of assistance with a complex situation that has not been adequately resolved through existing means. The Contractor shall receive cases for resolution from other Exchange/CMS sources, including, but not limited to, the

Exchange Call Center (via HICS or other prescribed means), the Division of Consumer Advocacy and Assister Support, Division of Small Business and Agent/Broker Innovation, Office of Communications, OPOLE and CMS/HHS leadership, and will have the capacity to both make outbound calls and receive inbound calls to resolve issues. The Contractor shall utilize a communications system that allows its agents to receive referrals in a variety of ways and shall ensure that the system also includes the capability for agents to communicate with issuers and/or consumers if further follow-up is needed to resolve cases. The Contractor shall be responsible for tracking assigned consumer cases and their status, recording instances and content of communications to and with issuers or consumers regarding the status of their case, and providing CMS with periodic updates about resolution response quality and operations.

The Contractor shall address referrals/cases using appropriate guidelines as determined by CMS. The Contractor shall ensure that its staff will receive appropriate training for assisting consumers that are experiencing complex eligibility and/or enrollment issues, with particular emphasis on the issues that are the most problematic for the Exchange Call Center and other entities to resolve (e.g., data matching issues [DMIs], delayed enrollment, retroactive terminations).

The Contractor shall also ensure that its staff receive in-depth training regarding the Exchange, eligibility and enrollment policies, and ensure that agents are able to successfully resolve complex cases with efficiency and sensitivity to the consumer. The Contractor shall maintain standard operating procedures (SOPs), and quick reference guides (QRGs). The Contractor shall provide a weekly report on metrics identified by CMS, including but not limited to inbound and outbound call volume; number of consumer cases; status of consumer cases (in progress, resolved, etc.), and number of cases with particular resolution types (referral, issuer action completed, etc.). The Contractor will also provide a monthly report to include key performance indicators (KPIs) identified by CMS which details the activity occurring under this task.

On a weekly basis, the Contractor will identify and communicate to CMS the cases they deem to require escalated support or CMS guidance to process/close. The average monthly size may ebb and flow as directed by CMS to reflect the spike and lull in overall consumer cases from month to month (e.g. to account for the seasonality of casework related to the annual Exchange Open Enrollment period).

The Contractor shall monitor and assess recurring case trends that require additional guidance or resolution. When patterns emerge across multiple issuers that can be addressed collectively through consistent direction or clarification, the Contractor shall develop recommended communications for CMS to share with the issuer community. Such communications may be disseminated through existing CMS channels, including weekly all-issuer webinars, all-issuer email blasts, and issuer-facing websites (e.g., REGTap and CMSzONE), to support more efficient resolution of escalated issues. Based on the above, on at least a monthly basis, the Contractor will derive from the consumer cases a report to CMS that identifies:

- The top 3-10 categories of common trends or shared root causes for the month's batch of consumer cases, including corresponding volume for each category.
- When appropriate, the top 3-5 scenarios or trends where additional CMS guidance or education can be delivered to the issuer community or to other stakeholders, including CMS Exchange partners, to more efficiently close out cases in questions and prevent future complex consumer cases.
- When appropriate, from the collective consumer cases:

- Identify volume of cases where the resolution could have been resolved by other means (such as by the FFE system, data reconciliation process, initial consumer contact with the Exchange, etc.) rather than through the Contractor.
- Identify and estimate volume by sub-category of resolution type.
- As applicable, identify volume of cases where the case was incorrectly categorized in HICS, broken down by incorrect categorization assignment.
- Identify volume and categorize instances of cases which appeared to require additional policy clarifications from CMS to be processed accordingly by the issuer or other stakeholder.
- Identify volume of cases where resolution appeared to indicate issuer did not comply with CMS issued regulations (such as enrollment transactions, change in circumstance (CIC), terminations, etc.). Identify corresponding top 3-10 areas of non-compliance that arise from the resolution narratives.
- Analyze cases to identify top 3-10 overall potential FFE system issues (apparent bugs or defects) that, if remedied, would have prevented issue from generating casework.
- Based on overall analysis, suggest top 3-10 best practices or communication tactics CMS should consider implementing to improve the assignment, processing and resolution of casework. For each, provide scenarios and example of appropriate resolutions based on CMS expectations and issued guidance.

3.5 Task 5: Updating the FFE

The Contractor shall update data in the FFE through the Batch Update Utility (BUU) process. Following the successful resolution of a dispute a BUU record is generated to identify the changes that need to be applied to the FFE database. The Contractor compiles the BUU records into a BUU file and performs pre-validation checks to ensure the updates will process successfully and are complete prior to submitting the BUU files to the Operational Analytics Contractor and the Quality Assurance (QA) Team for additional quality analysis. The Operational Analytics contractor converts the file into the appropriate format and submits it to the CMS contractor responsible for maintaining the FFE database. This process includes submitting BUU standard and non-standard updates. BUU files are generated in accordance with the Form 1095-A Correction Cycles (around 22 per year) and the Recon Run Calendar (monthly). Submit non-BUU “Hard Case” updates on the Form 1095-A Correction Cycle schedule.

The Contractor shall update policy cancelations for Rejected Enrollment and Undetected Overlap disputes in the FFE through the Update Policy API process. Following the successful resolution of a dispute that needs to be applied to the FFE database, the Contractor performs pre-validation checks to ensure the updates will process successfully and are complete prior to submitting the request to the FFE through the Update Policy API. The Contractor shall support development of additional updates through the Update Policy API process and other efforts to modernize the BUU process, as needed. The Contractor shall ensure that its processes include an appropriate audit trail mechanism (e.g., time/date stamping) so that actions taken can be tracked, reviewed and reported in a timely manner and prevent disputes from aging beyond established timeframes. There may be instances where the Contractor will need to manually review and process

erroneous electronic transactions to resolve those discrepancies due to the nature or level of the issues.

3.6 Task 6: Notice Generation

The Contractor will have the ability to generate automated notices which will in turn be printed and mailed to consumers by the Federal Marketplace Print contractor.

The Contractor shall generate the notices listed in Appendix B and other notices as identified by CMS. The Contractor shall design and generate new or updated notices as requested by CMS. The Contractor shall ensure that the notices generated meets or exceeds statutorily mandated requirements for translation into multiple languages as determined by CMS – this requirement may change throughout the course of the contract. Approximately 40 notices may need updating on a yearly basis.

Below is a summary of the tasks associated with notice generation:

- Securely send and received data (i.e. consumer names from CMS and CMS' contractors)
- Merge data into notice templates provided by CMS
- Securely send notice files through Electronic File Transfer to the print contractor
- Upload all consumer notices to the Document Storage and Retrieval System (DSRS)
- Provide CMS and the print contractor with reports about notice generation
 - The total number of notice generated and total page count of notices generated. Including a breakdown of number of notices generated by type and page count
 - When notices are generated
 - Additional ad hoc notice reports as requested by CMS
- Ensure notice files sent to the print contractor meet the print specifications
- Make any print vendor specification changes requested

3.7 Task 7: Telephone and Correspondence Support

The Contractor shall provide inbound and outbound telephone support to handle inquiries Monday-Friday from 8 a.m. Eastern Time to 8 p.m. EST to cover the majority of the time zones in the United States while ensuring statutorily-mandated language requirements are met or exceeded as determined by CMS. The Contractor shall provide support to consumers, issuers, FFE staff and SBEs.

The Contractor shall provide direct phone numbers with a voicemail box for calls and messages outside of business hours, on weekends, and during Federal holidays. The Contractor shall respond to voicemails within one business day.

The Contractor shall provide issuer call and email support through the ER&R Support Center by responding to calls and emails inquiries from issuers. The Contractor shall use the Enrollment Community to document issuer communications, including mass emails to issuers. In addition, the Contractor shall provide recommendations and testing support for technical enhancements to the Enrollment Community to support Enrollment Data Alignment.

The Contractor shall provide outbound call support for contacting issuers and consumers, particularly in cases where an outbound call could significantly expedite the resolution.

The Contractor shall provide additional outreach support as needed for the increase in Form 1095-A consumer disputes, UEs, and UPS cases.

Regarding consumer and issuer communications, the Contractor shall adhere to established scripting and SOPs approved by CMS. The representatives will need to be trained on these processes and address areas for process improvement on an as-needed ad hoc basis. The contractor is responsible to providing additional training to their representatives to stay current with updated direction or as part of process improvement.

The Contractor shall be responsible for recommending changes to existing scripts or creating new call scripts. All changes or new call scripts must be formally approved by the COR before implementation.

The Contractor will reach out to states and issuers to assist their resolution of issues. There may be requirements for cross coordination of functions, such as written correspondence, with the Exchange Call Center. As needed, the Exchange Call Center, which is managed by another government contractor, will assist applicants and enrollees to resolve their concerns and issues.

At any time, the Contractor may be asked to perform written correspondence support during enrollment and support activity processes to work through issue resolution and reconciliation issues. The Contractor shall use a CMS-provided system to record communications, generate responses to specific inquiries, and trigger formal decisions and planned actions due to changes reported by FFE, issuers, SBE, individuals, employers or other identified mechanisms.

In all aspects of telephone and correspondence support, the Contractor shall follow established SOPs to ensure that all correspondence is generated and tracked appropriately so that issuers, individuals, and SBEs can receive current and accurate information on the status of their correspondence.

3.8 Task 8: IT Systems

The Contractor shall access the Exchange infrastructure and Exchange-owned systems already developed to perform the required work. The Contractor will access multiple systems and services to research/resolve disputes and cases for issuer disputes, Form 1095-A consumer disputes, CCHC casework, and for implementation of other Tasks as applicable.

Infrastructure

Currently the CCIIO Enrollment Resolution and Reconciliation System (CERRS) and the Health Insurance Casework System (HICS) are part of the CMS Microsoft Azure Government enclave. The Marketplace Information Technology Group (MITG) procures Cloud computing resources through Office of Information Technology's (OIT) Cloud Computing Services (CCS). The services are divided among the following four (4) contractors:

Cloud Navigator Services (CNS)

- Provide onboarding and Program management support to new and existing teams.
- Implement Cloud Governance Structure and Strategy at Enterprise Cloud
- Collaborate and Manage Communications with Business Components and build communications portal

Cloud IT Operations & Support (ITOPS)

- Provide Cloud Operations and Onboarding to existing and new customers
- Build DevOps & DevSecOps Platform as a Service and Container as a Service Provide Managed services
- Cloud Automation of Shared Services for current and future state

Cloud Products & Tools (CPT)

- Procurement of compute resources for Cloud Service Providers (CSP) AWS, Microsoft Azure Government (MAG), Google, and SaaS service providers
- Procurement of labor for professional services for CSP
- Procurement and license management of cloud operations COTS tools for CMS Cloud

Cloud Security Operations

- Provide Network Operations Center/Security Operations Center (NOC/SOC) capabilities and work with teams to provide service
- Onboard Applications teams by providing Documentation and Operations support
- Help pilot and evaluate new security tools and work with Cloud Operations Teams

CMS will provide the necessary software licenses for operating and maintaining CERRS and HICS.

Exchange Systems

The following are Exchange systems that will assist in the research and resolution of issuer disputes, Form 1095-A consumer disputes and CCHC casework in addition to CERRS and HICS referenced in sections 3.8.1 and 3.8.2 along with source file information.

- **Marketplace Consumer Record (MCR)**
MCR provides Exchange Stakeholders with timely and relevant Exchange Information to resolve inquiries and discrepancies. MCR uses API technology to deliver search results. The Contractor will access MCR data through multiple APIs for research and resolution of disputes and casework.
- **Marketplace Change Utility (MCU)**
MCU allows users to search, view and download information about Exchange consumer insurance applications, insurance policies, plans and issuers.
- **Transaction Manager (TM)**
TM provides the user a view into the different phases of a member's enrollment history, including:
 - Initial eligibility request
 - Policy effectuation

- The person's association with the policy (subscriber, spouse, dependent, and other fields.)
- Policy cancellation information
- Financial data (total premium amount, individual premium amount, APTC, and other fields)
- **Multidimensional Insurance Data Analytics System (MIDAS)** A data analytics store that supports enrollments, applications, plans, and financial management data.

3.8.1 Subtask 8.1: CCHIO Enrollment Resolution and Reconciliation System (CERRS)

CERRS is the Microsoft Dynamics Customer Relationship Management (CRM) web application that serves as the technical solution for supporting issuer disputes, Form 1095-A consumer disputes and CCHC casework. Components include Microsoft Dynamics Customer Relationship Management (CRM) web application, and the CERRS Data Engine (CDE) which is the database that houses inbound source files from MIDAS, HICS and other sources.

The Contractor shall provide developer, programmer and operations support to maintain, enhance and develop CERRS for supporting issuer disputes, Form 1095-A consumer disputes and CCHC casework as described in this SOW. Additional support shall include the following:

- Support the enhancement process, including the fixing of system bugs, the evaluation of system enhancements prioritized by CMS, the development and testing of usability of improvements
- Provide 24/7 support for outages
- Prepare presentations and participate in CMS Technical Review Board meetings for CERRS and address any system findings and recommendations
- Support bi-weekly releases and Change Control Board meetings for CERRS, MCR. Enrollment Data Alignment CERRS Operations Reporting Environment (EDA CORE) and System of Exchange Enrollment Data (SEED)
- Develop updated versions of the Enrollment Dispute Form on a yearly basis
- Update and make changes to issuer dispute rules on a quarterly basis
- Refine casework system based on Exchange guidance
- Execute and monitor after hours data loads
- Execute and monitor ingestion of files, data remediation, cleansing of data and file scanning
- Collaborate across all Enrollment Data Alignment channels
- Support CMS documentation requirements
- Development for Ad-Hoc Work and Special Projects
- Support the MITG Independent Verification and Validation (IV&V) Assessment
- Participate in and support quarterly SAFE Agile PI Planning activities
- Support Enrollment Data Alignment special initiatives
- Re-engineer file processing
- Modernize code base to use Azure cloud base services, including coding activities
- Support CMS governance activities such as Pen Testing, ACT assessment, and external auditing

- Support CMS mandates and Marketplace laws and regulations
- Implement tools and processes allowing team to quickly and collaboratively deploy code following software engineering best practices like modularity, portability, CI/CD, and documentation, which may include coding activities
- Convert legacy or develop new reports on Power BI self-serve platform
- Enhance and expand data analytic capabilities
- Automate and streamline data management and maintenance
- Optimize Annual Readiness
- Optimize CRM solution
- Modernize automated testing and deployment process
- Participating as FedRAMP sponsor for the Microsoft Azure Government Snowflake deployment
- Disaster Recovery and Business Continuity Planning
- Integrate with CMS approved APIs for streamlined casework resolution and support
- Upgrading and modernizing systems as appropriate, including anticipated future software updates throughout the life of the contract.

The Contractor shall perform Dev-Ops (Development and Operations) associated with managing CERRS in the CMS MAG. This should include code escalation, monitoring, testing and evaluation of the implementation of changes and operations.

3.8.1.1 CERRS Marketplace Consumer Record (MCR)

CERRS MCR Search is an integrated component of CERRS. The Contractor shall maintain, develop and integrate with MCR and Enrollment Modernization APIs required for issuer disputes, 1095-A consumer disputes, CCHC and SEED as an MCR Data Contributor. The Contractor will contribute data related to actions taken and interactions unique to each consumer to MCR.

3.8.1.2 System of Exchange Enrollment Data (SEED)

SEED is an application hosted via the CMS Enterprise Portal that allows issuers and Third-Party Administrators (TPAs) to access real-time FFE data. The application offers a single location for viewing Health Insurance Oversight System (HIOS) specific enrollment data in real time, improving the issuers' ability to resolve casework and align their data with the FFE more efficiently than waiting to receive monthly reconciliation snap-shot files from the FFE. It also allows issuers to directly upload enrollment and payment disputes without having to set up EFT. SEED also allows issuers, and CMS users to submit and review prior-year dispute, post deadline inaccuracy dispute, and agent/broker update dispute approval requests.

The Contractor shall provide development and support for the integration of APIs that support issuer Enrollment Data Alignment and address functionality, and issuer requested items along with usability improvements. In addition, the Contractor shall manage the SEED approver and user process. The Contractor shall submit issuer approver requests to CMS for approval along with tracking and reporting SEED issuer approvers and users.

The Contractor shall provide developer and programmer support to maintain, enhance and develop SEED and its documentation. Development and programmer support for SEED should include the following:

- Support the enhancement process, including the fixing of system bugs identified by CMS and the user community, the evaluation of system enhancements prioritized by CMS, the development and testing of usability improvements, and the updating of issuer-facing technical documentation to reflect system changes
- Prepare 508-compliant issuer educational materials, slide presentations, and technical documentation. The contractor shall assume the need to produce four issuer-facing slide presentations per month outlining system updates and/or seeking input on potential system improvements, and two overhauls of comprehensive system technical documentation per year
- Regularly deliver weekly issuer-facing presentations on SEED updates, facilitate discussions with issuers to help prioritize the most valuable future enhancements, and engage in interactive question and answer (Q&A) session with issuers and users. The contractor shall assume presenting two half-hour sessions with a wide audience of issuers per month inclusive of interactive question and answer
- Conduct Monthly CMS/SEED Meetings
- Notify CMS and stakeholders of system outage/disruption
- Collaborate across all Enrollment Data Alignment channels
- Ensure all aspects of SEED and associated documentation are done in accordance with 508 Compliance
- The Contractor shall provide CMS quarterly recommendations on improvements to SEED including recommendations for new data elements and user interface improvements
- Monitoring and reporting on SEED performance
- Coordinate and schedule SEED releases
- Participate in and support quarterly SAFE Agile PI Planning activities

3.8.2 Subtask 8.2: Health Insurance Casework System (HICS)

HICS is the official tracking system for Exchange casework and is used by issuers that participate in the FFE and State Based Exchanges on the Federal Platform (SBE-FPs). The HICS system is how the Exchange Call Center and other stakeholders route cases that require investigation and resolution including the ability to examine Marketplace Consumer Record associated with those cases. The Contractor shall use the system to receive casework inquiries and other issues as directed by CMS.

In addition, the Contractor shall provide developer and programmer support to maintain HICS and its documentation. Developer and programmer support for HICS should include the following:

- Provide Tier 1, 2, and 3 Service Desk Support
- Enhance the system based on business needs and changing Marketplace laws and regulations
- Prioritize and coordinate functional releases on the program's bi-weekly sprint schedule
- Coordinate and implement bug fixes and reduce technical debt as needed

- Coordinate data integrations with Marketplace stakeholders such as NGD, NDW, MIDAS, EDL, EDA, SERCO, CERRS as needed
- Notify CMS and stakeholders of system outage/disruption
- Coordinate and schedule HICS Change Control Board meetings with appropriate stakeholders
- Prepare and present to the CMS Technical Review Board along with addressing any findings and recommendations
- 508 Compliance of HICS
- Monitoring and reporting on HICS performance
- HICS Disaster Recovery and Business Continuity Planning
- Support CMS for documentation requirements
- Support CMS governance activities such as PEN Testing, ACT assessment and external auditing
- Participate in and support quarterly SAFE Agile PI Planning activities

The Contractor shall perform Dev-Ops (Development and Operations) associated with managing HICS in the CMS MAG. This should include code escalation, monitoring, testing and evaluation of the implementation of changes and operations.

3.8.3 Subtask 8.3: Enrollment Data Alignment CERRS Operations Reporting Environment (EDA CORE)

The EDA CORE is a central repository that collocates a multitude of datasets pertaining to EDA. The application offers CMS personnel and contractors a holistic view of enrollment data and a shared reporting and analytics platform to more efficiently view issuer performance and process enrollment data reconciliation.

The Contractor shall maintain EDA datasets and improve the operational efficiency of the EDA process by avoiding file duplication and streamlining file and data transfer processes. This data will be stored in an easily accessible location where the data can be curated and organized to create reports, dashboards and other analytical processes.

The Contractor shall provide SME support to current and future stakeholders (including the CCIIO Marketplace Eligibility and Enrollment Group (MEEG) and PPFMG, 834 team, Recon Support, OPOLE Account Managers and Caseworkers, and others as needed). SME support includes but is not limited to answering user questions, offering technical assistance and performance tuning for SQL queries, and conducting training sessions. The Contractor shall improve the efficient cooperation among multiple stakeholders to view and update files across all EDA inputs and channels. The Contractor is responsible for collaborating internally among teams supporting other Project Tasks, including teams who are supporting Forms 1095-A operations as well as with any applicable external stakeholders as part of implementation of the core functions.

The Contractor shall develop and maintain functionality to distribute the Enrollment Alignment Performance Summary (EAPS) reporting to issuers and CMS stakeholders each month as well as Account Manager dashboard reporting. The Contractor shall identify, propose, test, and implement enhancements to this reporting periodically. The Contractor shall provide SME

support to CMS and answer issuer questions regarding EAPS reporting. In addition, the Contractor shall support ongoing Account Manager Dashboard development.

The Contractor shall ingest high volumes of complex 834 data to the EDA CORE environment. This includes obtaining extracts of all required historical 834 data and loading into the EDA CORE, loading all new 834 transactions daily, providing supporting channels for end users, and developing 834 reports to support 834 operations. During Open Enrollment, the Contractor should be prepared to handle thousands of transactions per hour.

The Contractor should develop and maintain functionality for developing payment anomaly, enrollment data alignment, and dispute processing reports. This includes updating the reports monthly as new data arrives and summarizing data findings for CMS SMEs. It also includes working with CMS to develop requirements for new reports and updating existing requirements.

The Contractor shall provide developer and programmer support to maintain, enhance and develop EDA CORE and its documentation. Development and programmer support for EDA CORE should include the following:

- Support the enhancement process, including the fixing of system bugs, identified by CMS and the user community, the evaluation of system enhancements prioritized by CMS, the development and testing of usability improvements and the updating of documentation to reflect system changes
- Support CMS governance activities such as PEN (penetration) testing, ACT (Assessment Certification, and Testing) assessment, and external auditing
- Support disaster recovery and business continuity planning
- Participate and support quarterly SAFE Agile PI Planning activities
- Prepare 508-compliant educational materials, slide presentations, and technical documentation
- Optimally develop and migrate data
- Enhance and expand data analytic capabilities
- Add new datasets, as needed, to support ongoing EDA efforts
- Notify CMS and stakeholders of system outages and disruptions
- Provide Tier 2 and 3 Service Desk support
- Collaborate across all EDA channels
- Provide, support, and facilitate UAT (User Acceptance Testing) for requested changes

3.9 Task 9: Standard Operating Procedures (SOPs) Business Requirements and Quality Assurance

CMS will provide the Contractor with a set of developed SOPs (sample list of SOPs are included in Appendix C). These SOPs will provide guidance to the Contractor regarding the performance of tasks described in this SOW. As the Contractor executes activities, it is expected that the Contractor shall draft specific and actionable recommendations about ways to improve workflow processes.

The Contractor shall also be responsible for recommending changes or creating additional SOPs to align with changes in business requirements or processes. SOPs should be updated on an ongoing monthly basis as needed. The Contractor's recommendations will be reflected in any documentation used to notify staff of any necessary or unplanned procedural changes, as well as in revised/drafted SOPs. All recommended changes or new SOPs must be formally accepted and approved by the COR.

In addition, CMS will provide the Contractor with a set of developed user guides and job aids (sample user guides and job aids included in Appendix D). The Contractor shall be responsible for modifying these instructions and creating instructions for new tasks or work products as necessary. User guides and job aids should be reviewed at a minimum on a quarterly basis and updated accordingly.

Performance, Quality Assurance, and Data Analysis

The Contractor shall create a systematic quality management program that encompasses:

- **Performance/Quality Assurance** - refers to the systems in place to ensure the quality standards and measurable service levels of rendered services are established. In addition, these systems ensure that process improvement efforts are set in place to continually refine and improve services and comply with any established metrics or applicable quality assurance surveillance plan.
- **Data Collection and Analysis** - ensures that performance data is continually collected and analyzed. Data is used to monitor program integrity and to continuously identify areas for process improvement. Once areas have been identified for improvement, mechanisms will be put into place to focus on strengthening such processes.

More information on requirements for these areas is described below:

- **Quality Assurance, Improvement and Control Plan**
The Contractor shall design, plan, and deliver a Quality Assurance Plan within forty-five (45) days of the start of the Base Period. The plan shall include procedures for evaluation of performance using the Key Performance Indicators (KPIs) described below. This plan shall include:
 - a) Schedule of quality assurance tasks and responsibilities
 - b) Methodology for documenting standard practices
 - c) Approach for process reviews and audits
 - d) Testing of ideas or methods to improve identified processes
 - e) Risk and root cause identification and tracking
 - f) Issue identification and tracking
 - g) Mitigation planning, implementation, and resolution documentation
- **Business Process Improvement**
The Contractor shall prepare and deliver business process improvement recommendations in a Monthly Business Process Improvement Report. The report shall recommend any improvements to the enrollment and resolutions/reconciliation systems and operating procedures based on the Key Performance Indicators (KPIs) established.

- **Object and Key Results (OKRS)**

CMS will provide an initial set of OKRs and related KPIs that include quantitative benchmarks and service levels associated with the contract. The Contractor shall develop additional quantitative and qualitative benchmarks and service levels for areas not addressed by CMS. The Contractor shall ensure that the KPIs are specific, measurable, and reliable. The KPIs developed by the Contractor shall focus on enhancing, evaluating and improving the efficiency and reporting of ER&R functions for the Exchange. The Contractor shall develop these KPIs via industry research, best practices, and innovations such as time studies/analyses. The KPIs will be presented, refined and approved by CMS at a time to be determined during the business process improvement kick-off. The Contractor shall report on established KPIs in the Monthly Performance Report.

- **Continuous Quality Improvement**

The Contractor shall prepare a monthly quality assurance report that contains quality reports to continuously improve processes and performance indicators. The reports must evaluate and recommend refined processes that address issues and create greater efficiencies.

- **Data Collection, Analysis and Trending**

The Contractor shall receive data at the issuers and exchanges level from the FMPS and the Operations Analytic Contractor on enrollment process related issues and transactions to perform analysis and report key findings as determined by CMS for program tracking, program integrity, and performance improvement by at least the following elements:

- a) Type of entity submitting documentation (e.g., QHP, SBE)
- b) Correct enrollee information i.e. DOB sex marital status address etc.
- c) Reason codes - for submission (e.g., inconsistency period, special enrollment period, termination, cancelation, initial enrollment, etc.)
- d) Effective dates
- e) Terminations
- f) Cancellations
- g) Correct plan level
- h) APTC/CSR validations
- i) Comparison of ASC X12N 834 and ASC X12N 820 electronic transactions and relevant reconciliation reports
- j) Error Reports
- k) Correction of Errors
- l) Reporting of Errors – to include (number of Errors, types of Errors, patterns of issuers, patterns directly related to issuer.
- m) Trending analysis of enrollment reconciliation, enrollment resolution, correction of errors
- n) Source and content of common issues and inquiries
- o) Identified areas of vulnerability for enrollment, resolution, reconciliation, maintenance, APTC, CSR, Errors and corrections
- p) Additional elements as identified by CMS

Data collection shall include, at a minimum, consolidated weekly, monthly and annual reports. The Contractor shall provide ad hoc reports as designated by CMS.

3.10 Task 10: Ad Hoc Work and Special Projects

The Contractor shall prepare Special Projects as designated by CMS. These activities shall include, but are not limited to, the following:

1. Conducting short studies, special projects, or data analyses as directed by the COR. The Contractor may be directed to provide support for additional tasks supporting Marketplace operations as defined by CMS throughout the Period of Performance depending on evolving business needs and funding availability.
2. Creating and providing training sessions, workshops, and innovative technology or other related topics for stakeholders on an ad hoc basis as directed by CMS.
3. Providing program subject matter expertise and related support for the annual revision of the FFE Enrollment Manual to incorporate regulatory changes and updates, such as the Marketplace Stabilization Rule and annual “*Letter to Issuers in the Federally-Facilitated Exchanges.*” The process is broken down into four (4) phases: Preparation Phase, Edit/Review Phase, Clearance Phase, and Education and Publication Phase. The timeline for this project is anticipated to run from January-July but is subject to the direction of CMS. The Contractor shall provide the following support:
 - Oversee Enrollment Manual Project Plan
 - Manage stakeholder engagement with CCHIO resources
 - Facilitate stakeholder meetings including kick-off meetings, weekly stand-up meetings and SME meetings
 - Manage deliverables including: tracking of enrollment manual iterations, incorporation of SME feedback, and approval submissions
4. The Contractor shall provide support for the Death Periodic Data Matching (PDM) project which is the process of identifying consumers that have become deceased during the coverage year and removing them from coverage. CMS identifies consumers through a data matching process between Exchange enrollees and the stored list of deceased persons provided by the Social Security Administration (SSA). For consumers found to be deceased, initial warning notices are mailed to consumers and consumers are given thirty (30) days to respond to the notice if they disagree with the Exchange’s findings and want to attest that they are not deceased. If consumers do not respond, they are automatically removed from coverage. Currently the Exchange only sends notices and takes action on single-member applications, although it aims to include multi-member applications when systems allow. Consumers who wish to respond to an initial warning notice by attesting they are not deceased must call a voicemail hotline to provide this attestation. Consumers then receive a response notice in the mail confirming receipt of their attestation and their removal from Death PDM processing. When notifying single-member applications, approximately 5,000 initial warning notices are sent and CMS expects less than 1% of consumers to attest to not being deceased. It is estimated that

over 99% of identified consumers who receive generated initial warning notice are in fact deceased and the Exchange should not expect to receive a response from these consumers.

Death PDM will be conducted twice a year. The Contractor shall provide support for Death PDM consumer response operations, including the following activities:

- Provide telephone support with a toll-free telephone number capable of receiving voicemails and making outbound calls, including TTY services
 - Generate notices confirming receipt of attestation to consumers on an ongoing basis as calls are received
 - Record consumers who call-in and are successfully matched to the list of notified deceased consumers, and report back a running list of these consumers to the FFE through email, weekly, upon commencement of the telephone line being open.
5. The contractor shall provide support for a functional Employer Notice Helpdesk on an ongoing basis, to aid employers who reach out with questions regarding Employer Notices sent by the FFE. The Contractor shall provide support for Employer Notice Call Center operations, including the following activities:
- Coordinate with CMS staff (including CCIIO and the Office of Hearings and Inquiries (OHI) for appeals-related aspects)) and contractor teams on timeline for launch of Employer Notice Call Center, modifications to Call Center scripts, updates to notice content, or any other changes impacting Call Center messaging/operations.
 - Provide telephone support
 - Host a toll-free telephone number (inclusive of TTY services) capable of receiving real-time incoming calls and voicemails, making outbound calls, and transferring calls (e.g., to employer appeals call center).
 - Develop and implement a Call Center script that is reviewed and approved by CMS.
 - Use an existing system solution (preferred) or develop and implement a system solution for organized call and issue triage and tracking.
 - Answer calls and respond to voicemails.
 - Hours of operation: Monday through Friday from 8 AM – 8 PM EST, excluding federal holidays.
 - Provide written weekly, monthly, and annual reports related to Call Center performance metrics and issue tracking; reports to be available upon launch of the Call Center, to be provided as needed or at a frequency determined by the COR.
6. Other projects related to the scope of the SOW inclusive of all Tasks.

3.11 Task 11: Transition-Out (Optional)

At the end of the contractual performance period (base period or any option period exercised), CMS may, with thirty (30) days' notice to the incumbent contractor, transition work activities from the contractor to an incoming contractor(s) designated by CMS. CMS may exercise this optional task at its discretion. This task describes the work that shall be done by the incumbent

contractor to enable the incoming contractor to assume all responsibilities for this acquisition from the incumbent. During such transition efforts, the incumbent contractor shall provide access to work products and support a smooth, orderly, and cooperative transition of associated IT environments and services. The incumbent contractor shall minimize risks to operations and ensure complete transfer of responsibilities. All operational data required by the incoming contractor or requested by CMS shall be transitioned in a timely manner, while any remaining data and associated IT environments are to be destroyed according to CMS Data Security requirements.

All transition activities must be completed by the incumbent contractor and incoming contractor within a period that begins 3 months prior to the end of the incumbent contract and not to exceed 3 months. Transition requires the incumbent contractor to:

1. Work with the incoming contractor to transition any applicable activities
2. Participate in transition meetings, as necessary, with the incoming contractor
3. Continue supporting all tasks until they are transitioned to the incoming contractor per a Transition Plan approved by CMS
4. Respond to any inquiries until the end of the designated transition period
5. Receive and forward electronic and paper documentation to the incoming contractor no later than the end of the designated transition period
6. Identify and perform any other tasks required for a successful transition.

The incumbent Contractor shall establish a Joint Operating Agreement (JOA) with the incoming contractor within 7 calendar days of award of the new contract or as directed by the COR. Not more than one (1) week after notification by CMS that transition to the incoming contractor will take place, the incumbent contractor shall submit to the COR a draft written JOA for CMS approval. The purpose of the JOA is to establish a process for managing and executing the workload while both contracts are in place and to also establish a process to fully transition the workload from the incumbent contractor to the new contractor. The JOA shall illustrate the way the two entities will maintain support during the transition of the work from the incumbent's task order to the incoming task order including methods that will be used to communicate and coordinate activities and to communicate to CMS. The JOA shall be delivered by the incumbent contractor and reviewed and approved by the COR.

The incumbent contractor shall also provide to CMS and the incoming contractor all items developed under this task order (where applicable), including, but not limited to:

1. All CMS systems development life cycle (SDLC) project artifacts
2. Reports, copybooks, test case scenarios and other related products not specifically described in this document
3. All system and business requirement documentation, including SOPs, developed under this contract
4. Data models
5. Draft and Final Closeout Reports
6. Any additional items to be identified by the COR

The incumbent contractor shall provide draft and final closeout reports to CMS prior to the end of the task order period of performance. The incumbent contractor shall provide CMS with a

draft closeout report six (6) weeks prior to the end of the task order period of performance at a date specified by CMS. CMS will provide written comments on the draft closeout report within two (2) weeks of receipt of the draft closeout report, and the incumbent contractor shall provide a final closeout report to at least CMS two (2) weeks prior to the end of the task order period of performance. The final closeout report shall incorporate CMS comments. The incumbent contractor shall provide the closeout report in the manner specified by CMS, to include specifications on content and format.

3.12 Task 12: Transition-In (Optional)

Certain tasks under this SOW are currently performed by an incumbent Contractor, which will require a 3-month period for transferring responsibilities and relevant information and technology data to an incoming contractor. This task describes the work that shall be done by the incoming contractor to assume all responsibilities for this acquisition from the incumbent.

To ensure a coordinated transition, the incoming contractor shall sign a JOA with the incumbent contractor on receipt of the agreement from the incumbent and within seven (7) calendar days from the date of the start of the Base Period of this task order or as directed by the COR. The JOA, developed by the incumbent and approved by the COR, shall establish a process for managing the workload while both task orders are in place and to also establish a process to fully transition the workload from the incumbent contractor to the incoming contractor. The JOA shall illustrate the way the two entities will maintain support during the transition of the work from the incumbent's task order to the new task order including methods that will be used to communicate and coordinate activities and to communicate with CMS.

To plan, monitor, and manage a coordinated transition per this task, the incoming contractor shall work collaboratively with the incumbent contractor and provide to the COR for approval an initial draft of a Transition Plan for this task by e-mail within eight (8) business days from the award date of this optional task. This plan, which shall be integrated with the Project Management Plan for the entire SOW as detailed in Task 1 above. The Transition Plan shall include a Microsoft Project Plan or an alternative format agreed upon between CMS and the contractor to outline development milestones and shall address all the tasks and subtasks in this SOW that will transfer. The incoming contractor shall identify deliverables and milestones to ensure an orderly and complete transition. This activity will allow the incoming contractor to assume full operating accountability and responsibility and minimize the impact of the transition to CMS's business and operations.

Any agreed-upon Transition Management Plan shall be approved by CMS and shall include, but not be limited to, the following:

1. Description of transition activities
2. In-depth Transition Schedule, including key milestone events and activities. This shall include a detailed timeline for work in progress and production phasing and cutovers due to the transition to the incoming contractor
3. Comprehensive listing of the responsibilities of all personnel participating in the transition to include the policies, practices and procedures to be employed by the incumbent contractor to ensure there is no conflict between any transitional activities

4. Complete and detailed resource-planning/resource-turnover analysis (e.g. plans for onboarding of new contractor staff, plans to ramp-down incumbent contractor staff, any outstanding technical or contractual issues that need to be resolved)
5. Inventory management of PII, Government Funded Equipment (GFE), Government Furnished Information (GFI) and contractor owned resources
6. Transfer of, and access to, in-process deliverables and work products. In addition, updated inventory lists and updated deliverables list
7. Transfer of licenses and warranties
8. Transition risk management plan that includes a list of the potential risks during the transition period and the plan to mitigate each
9. Communications and reporting plan during the transition period
10. Performance metrics
11. Attaining Authorization to Operate (ATO)
12. Any other information either deemed necessary by the contractor or requested by CMS

4. Deliverables Schedule

CMS, through the COR, will review and approve all work products and deliverables submitted by the Contractor. Acceptance of deliverables will be based on an assessment of accuracy, clarity, timeliness, sufficient detail, and completeness. Deliverables shall be well-developed, well-written, and cover all aspects of the requirements of the deliverable. Any questions on the technical quality of the deliverables shall be addressed to the Contractor within a time mutually agreed upon between the Contractor and the COR.

The following is a schedule of formal deliverables corresponding to the SOW tasks. To streamline the reporting of recurring items with the same delivery frequency, the Contractor may, with approval of the COR, incorporate individual deliverable items into a consolidated deliverable report. All deliverables must be formally accepted by CMS in accordance with the SOW and/or inspection and acceptance provisions of the contract/task order. Deliverables not meeting the requirements of the contract will be rejected by CMS. Deliverables updated after initial submission shall also be delivered to CMS. Revisions and updates to the schedule of deliverables may be required in the Base Period.

All written deliverables shall be clearly written in plain English, and use concise, grammatically correct language. All materials must be reviewed and approved by CMS, and, where necessary, meet CMS guidelines. Upon direction by CMS, this may include the preparation of 508-compliant materials. The Contractor shall proofread and correct, as appropriate, all written deliverables for this task order, regardless of whether the deliverables are in draft or final version prior to delivery to CMS. Where verbatim text from non-Contractor sources have been used, the Contractor shall cite or otherwise attribute such text in the deliverable.

The Contractor shall perform activities and produce deliverables covering the functional areas of support identified within this SOW. The deliverables shall follow the format as agreed upon between CMS COR and contractor staff. The deliverables shall contain information that demonstrates the understanding of the scope and tasks described above. The Contractor shall work with CMS to produce interim products as necessary to support timely identification, evaluation and recommendation on all project related deliverables to ensure that the technical

solutions provided by the Contractor fully satisfy the requirements presented in the Business Case Analysis and this SOW.

Table 5: SCHEDULE OF DELIVERABLES

Item Description	SOW Reference	Delivery Date / Frequency	Recipient	Base / Option Period Delivery
Form 1095-A Consumer Disputes and Enrollment Process Improvement Report	3.1	Monthly	COR	Base Period + Option Period(s)
Initial Project Implementation and Operations Plan (PIOP)	3.1	Within 7 business days following the start of the Base Period; to be updated quarterly or ad hoc as needed	COR	Base Period + Option Period(s)
Initial Contract Kickoff Meeting	3.1	Within 12 business days after the start of the base period unless otherwise directed	COR	Base Period
Monthly Progress, Data and Cost Report	3.1	Monthly – third Monday of each month, 2 days prior to Budget Review Meeting	COR	Base Period + Option Period(s)
Periodic In-Person Meetings with CMS	3.1	At least 4 meetings per year or as directed by the COR	COR	Base Period + Option Period(s)
Weekly Meeting Deliverables	3.1	Weekly; Agenda 1 day prior	COR	Base Period + Option Period(s)
Other Meeting Deliverables	3.1	Ad hoc	COR	Base Period + Option Period(s)

Item Description	SOW Reference	Delivery Date / Frequency	Recipient	Base / Option Period Delivery
Task Order Summary Report	3.1	Draft delivered 10 th month of final task order year and submit a final draft 30 days after the task order ends	COR	Final Period
Issuer Outreach Tracker Report	3.2	Monthly, upon commencement of operations	COR	Base Period + Option Period(s)
Payment and Enrollment Dashboard	3.2	Weekly, upon commencement of operations	COR	Base Period + Option Period(s)
Issuer Disputes	3.2	Daily and in accordance with the Service Level Agreement (SLA)	COR	Base Period + Option Period(s)
Reports and dashboards	3.2.2	Monthly	COR	Base Period + Option Period(s)
Technical assistance calls with issuers	3.2.2	3-4 calls monthly	COR	Base Period + Option Period(s)
CERRS Distribution by Coverage Year and Case Type	3.3	Daily, Weekly and Monthly	COR	Base Period + Option Period(s)
1095-A Corrections Report (1095-A Metrics Report)	3.3	Weekly, upon commencement of operations	COR	Base Period + Option Period(s)
Processing Form-1095A Incoming Consumer Disputes, Errors/Issuers Transactions	3.3	Daily and in accordance with the Service Level Agreement (SLA)	COR	Base Period + Option Period(s)
Bi-weekly full CCHC Report (Complex Case Periodic Data Analysis & Quality Report)	3.4	Bi-weekly	COR	Base Period + Option Period(s)

Item Description	SOW Reference	Delivery Date / Frequency	Recipient	Base / Option Period Delivery
CCHC Root Cause Report (Complex Case Monthly Report)	3.4	Monthly. NLT 15 th of each month	COR	Base Period + Option Period(s)
Complex Case Standard Operating Procedures	3.4	As directed by COR	COR	Base Period + Option Period(s)
CCHC Assister Referral Case Metrics	3.4	Monthly. NLT 15 th of each month	COR	Base Period + Option Period(s)
CCHC Quarterly Root Cause Analysis Report	3.4	Quarterly	COR	Option Period(s)
BUU Correction Cycle (FFE Updates Report)	3.5, 3.3.3	Bi-weekly/Monthly	COR	Base Period + Option Period(s)
Monthly Manual Generation Metrics	3.5	Monthly	COR	Base Period + Option Period(s)
Notice Generation	3.6	Ad hoc	COR	Base Period + Option Period(s)
Telephone and Correspondence Support	3.7	Monday – Friday 8 AM – 8 PM Eastern Standard Time	COR	Base Period + Option Period(s)
ERR1095A Operations and Maintenance	3.8	Yearly	COR	Base Period + Option Period(s)
ERR1095A Functional Requirements	3.8	Yearly	COR	Base Period + Option Period(s)
CERRS Interface Control Document	3.8.1	Yearly	COR	Base Period + Option Period(s)
CERRS Database Design Document	3.8.1	Yearly	COR	Base Period + Option Period(s)

Item Description	SOW Reference	Delivery Date / Frequency	Recipient	Base / Option Period Delivery
CERRS High-Level Functional Design Document	3.8.1	Yearly	COR	Base Period + Option Period(s)
CERRS System Design Document	3.8.1	Yearly	COR	Base Period + Option Period(s)
CERRS RTM	3.8.1	Yearly	COR	Base Period + Option Period(s)
CERRS Testing SOP	3.8.1	Yearly	COR	Base Period + Option Period(s)
System of Exchange Enrollment Data (SEED)	3.8.1.2	Daily, Weekly, and Monthly as determined by the SOW	COR	Base Period + Option Period(s)
Enrollment Data Alignment CERRS Operations Reporting Environment (EDA CORE)	3.8.3	Daily	COR	Base Period + Option Period(s)
Business Process Improvement Report	3.9	At Kickoff Mtg and monthly	COR	Base Period + Option Period(s)
Monthly Performance Report	3.9	At Kickoff Mtg and monthly	COR	Base Period + Option Period(s)
Quality Assurance Report	3.9	Monthly, upon commencement of operations	COR	Base Period + Option Period(s)
SOPs and Business Requirements/User Guides/Job Aids	3.9	Monthly/Quarterly updates as needed	COR	Base Period + Option Period(s)
Enrollment Manual	3.10	Annually or as needed	COR	Base Period + Option Period(s)
Training and Workshops	3.10	Ad hoc	COR	Base Period + Option Period(s)

Item Description	SOW Reference	Delivery Date / Frequency	Recipient	Base / Option Period Delivery
Death Period Data Matching Project	3.10	Weekly	COR	Base Period + Option Period(s)
Employer Notice Helpdesk	3.10	Monday – Friday 8 AM – 8 PM Eastern Standard Time	COR	Base Period + Option Period(s)
Ad Hoc and Special Projects Reports	3.10	Weekly, Monthly, Annually, as required Up to 10 ad hoc reports	COR	Base Period + Option Period(s)
Final Closeout Report	3.11	At least 2 weeks prior to the end of the task order period of performance – if exercised	COR	Final Period
Joint Operating Agreement	3.11, 3.12	7 calendar days of start of Base Period – if exercised	COR	Base Period
Transition Management Plan	3.12	8 days after award of Optional Task – if exercised	COR	Base Period

5. Inspection and Acceptance

The Contractor shall meet the critical performance objectives of the services being acquired, including the required performance threshold(s) for those objectives. Failure to satisfactorily perform may result in re-performance, reduction in compensation, or other remedies afforded to the government through the task order or by law.

6. CMS Security and Privacy

Information Security and/or Physical Access Security

6.1 Baseline Security Requirements

- a. **Applicability.** The requirements herein apply whether the entire contract or modification (hereafter "contract"), or portion thereof, includes either or both of the following:
- i. **Access (Physical or Logical) to Government Information:** A Contractor (and/or any subcontractor) will have or will be given the ability to have, routine physical (entry) or logical (electronic) access to government information.
 - ii. **Operate a Federal System Containing Information:** A Contractor (and/or any subcontractor) will operate a federal system and information technology containing data that supports the HHS mission. In addition to the Federal Acquisition Regulation (FAR) Subpart 2.1 definition of "information technology" (IT), the term as used in this section includes computers, ancillary equipment (including imaging peripherals, input, output, and storage devices necessary for security and surveillance), peripheral equipment designed to be controlled by the central processing unit of a computer, software, firmware and similar procedures, services (including support services), and related resources.
- b. **Safeguarding Information and Information Systems.** All government information and information systems must be protected in accordance with HHS/CMS policies and level of risk. At a minimum, the Contractor (and/or any subcontractor) must:
- i. Protect the:
 - **Confidentiality**, which means preserving authorized restrictions on access and disclosure, based on the security terms found in this contract, including means for protecting personal privacy and proprietary information;
 - **Integrity**, which means guarding against improper information modification or destruction, and ensuring information non-repudiation and authenticity; and
 - **Availability**, which means ensuring timely and reliable access to and use of information.
 - ii. Categorize all information owned and/or collected/managed on behalf of HHS/CMS and information systems that store, process, and/or transmit HHS information in accordance with FIPS 199 and National Institute of Standards and Technology ([NIST Special Publication \(SP\) 800-60, Volume II: Appendices to Guide for Mapping Types of Information and Information Systems to Security Categories](#)). Based on information provided by the ISSO, CISO, CMS SOP, or other representative, the impact level for each Security Objective (Confidentiality, Integrity, and Availability) and the Overall Impact Level, which is the highest watermark of the three factors of the information or information system are the following:
 - **Confidentiality:** ☐ Low ☒ Moderate ☐ High
 - **Integrity:** ☐ Low ☒ Moderate ☐ High
 - **Availability:** ☐ Low ☒ Moderate ☐ High
 - **Overall Impact Level:** ☐ Low ☒ Moderate ☐ High

- iii. Based on the agreed-upon level of impact, implement the necessary safeguards to protect all information systems and information collected and/or managed on behalf of HHS/CMS regardless of location or purpose.
- iv. Report any discovered or unanticipated threats or hazards by either the agency or contractor, or if existing safeguards have ceased to function immediately after discovery, **within one (1) hour or less**, to the government representative(s).
- v. Adopt and implement all applicable policies, procedures, controls, and standards required by the HHS/CMS Information Security Program to ensure the confidentiality, integrity, and availability of government information and government information systems for which the Contractor is responsible under this contract or to which the Contractor may otherwise have access under this contract. Obtain all applicable security and privacy policies by contacting the CO/COR or HHS/CMS security and/or privacy officials.
- c. **Privacy Act.** Comply with the Privacy Act requirements (when applicable), and tailor FAR and HHSAR clauses as needed.
- d. **Privacy Compliance.** Comply with the E-Government Act of 2002, NIST SP 800-53, and applicable HHS/CMS privacy policies, and complete all the requirements below:
 - i. Per the Office of Management and Budget (OMB) Circular A-130, Personally Identifiable Information (PII), is "information that can be used to distinguish or trace an individual's identity, either alone or when combined with other information that is linked or linkable to a specific individual." Examples of PII include, but are not limited to the following: Social Security number, date and place of birth, mother's maiden name, biometric records, etc.
 - ii. Based on information provided by the ISSO, system/data owner, or other security or privacy representative, it has been determined that this solicitation/contract involves:
☐ No PII ☒ PII
 - iii. The Contractor must support the agency with conducting a Privacy Threshold Analysis (PTA) for the information system and/or information handled under this contract to determine whether or not a full Privacy Impact Assessment (PIA) needs to be completed.
 - If the results of the PTA show that a full PIA is needed, the Contractor must support the agency with completing a PIA for the system or information within *[CMS to insert contract-specific timeline]* after completion of the PTA and in accordance with HHS policy and OMB M-03-22, *Guidance for Implementing the Privacy Provisions of the E-Government Act of 2002*.
 - The Contractor must support the agency in reviewing the PIA at least every **three years** throughout the system development lifecycle (SDLC)/information lifecycle, or when determined by the agency that a review is required based on a major change to the

system, or when new types of PII are collected that introduces new or increased privacy risks, whichever comes first.

- e. **Controlled Unclassified Information (CUI). Executive Order 13556** defines CUI as "information that laws, regulations, or Government-wide policies require to have safeguarding or dissemination controls, excluding classified information." The Contractor (and/or any subcontractor) must comply with *Executive Order 13556, Controlled Unclassified Information, (implemented at 3 CFR, part 2002)* when handling CUI. 32 C.F.R. 2002.4(aa) As implemented the term "*handling*" refers to "...any use of CUI, including but not limited to marking, safeguarding, transporting, disseminating, re-using, and disposing of the information." 81 Fed. Reg. 63323. The requirements below apply only to nonfederal systems that process, store, or transmit CUI, or that provide security protection for such components. All sensitive information that has been identified as CUI by a regulation or statute, handled by this solicitation/contract, must be:
 - i. Marked appropriately;
 - ii. Disclosed to authorized personnel on a Need-To-Know basis;
 - iii. Protected in accordance with NIST SP 800-53, *Security and Privacy Controls for Information Systems and Organizations* applicable baseline if handled by a Contractor system operated on behalf of the agency, or NIST SP 800-171, *Protecting Controlled Unclassified Information in Nonfederal Information Systems and Organizations* if handled by internal Contractor system; and
 - iv. Returned to HHS control, destroyed when no longer needed, or held until otherwise directed. Information and/or data must be disposed of in accordance with NIST SP 800-88, *Guidelines for Media Sanitization*.
- f. **Protection of Sensitive Information.** For security purposes, information is *or* may be sensitive because it requires security to protect its confidentiality, integrity, and/or availability. The Contractor (and/or any subcontractor) must protect all government information that is or may be sensitive by securing it with a solution that is validated with current FIPS 140 validation certificate from the NIST CMVP.
- g. **Confidentiality and Nondisclosure of Information.** Any information provided to the contractor (and/or any subcontractor) by HHS or collected by the contractor on behalf of HHS must be used only for the purpose of carrying out the provisions of this contract and must not be disclosed or made known in any manner to any persons except as may be necessary in the performance of the contract. The Contractor assumes responsibility for protection of the confidentiality of Government records and must ensure that all work performed by its employees and subcontractors must be under the supervision of the Contractor. Each Contractor employee or any of its subcontractors to whom any HHS records may be made available or disclosed must be notified in writing by the Contractor that information disclosed to such employee or subcontractor can be used only for that purpose and to the extent authorized herein.

The confidentiality, integrity, and availability of such information must be protected in accordance with HHS and CMS policies. Unauthorized disclosure of

information will be subject to the HHS/CMS sanction policies and/or governed by the following laws and regulations:

- i. 18 U.S.C. 641 (Criminal Code: Public Money, Property or Records);
 - ii. 18 U.S.C. 1905 (Criminal Code: Disclosure of Confidential Information);
and
 - iii. 44 U.S.C. Chapter 35, Subchapter I (Paperwork Reduction Act).
- h. **Internet Protocol Version 6 (IPv6).** All procurements using Internet Protocol must comply with OMB Memorandum M-05-22, *Transition Planning for Internet Protocol Version 6 (IPv6)*.
- i. **Information and Communications Technology (ICT).** ICT products and services from prohibited entities/sources must not be used/acquired in compliance with Public Law 115-232, Section 889 Parts A and B, FAR 4.21, FAR 52.204.23, FAR 52.204.24, and FAR 52.204.25. The contractor (and/or any subcontractor) must notify the government if they identify prohibited ICT products and/or services are used during the contract performance.
- j. **Government Websites.** All new and existing public-facing government websites must be securely configured with Hypertext Transfer Protocol Secure (HTTPS) using the most recent version of Transport Layer Security (TLS). In addition, HTTPS must enable HTTP Strict Transport Security (HSTS) to instruct compliant browsers to assume HTTPS at all times to reduce the number of insecure redirects and protect against attacks that attempt to downgrade connections to plain HTTP. For internal-facing websites, HTTPS is not required, but it is highly recommended. Consult the *HHS Policy for Internet and Email Security* for additional information.
- k. **Contract Documentation.** The Contractor must use provided templates, policies, forms and other agency documents found at <https://www.cms.gov/Research-Statistics-Data-and-Systems/CMS-Information-Technology/InformationSecurity/Information-Security-Library> to comply with contract deliverables as appropriate.

Standard for Encryption. The Contractor (and/or any subcontractor) must:

- i. Comply with the *HHS Standard for Encryption of Computing Devices and Information* to prevent unauthorized access to government information.
- ii. Encrypt all sensitive federal data and information (i.e., PII, protected health information [PHI], proprietary information, etc.) in transit (i.e., email, network connections, etc.) and at rest (i.e., servers, storage devices, mobile devices, backup media, etc.) with encryption solution that is validated with current FIPS 140 validation certificate from the NIST CMVP.
- iii. Secure all devices (i.e.: desktops, laptops, mobile devices, etc.) that store and process government information and ensure devices meet HHS and CMS-specific encryption standard requirements. Maintain a complete and current inventory of all laptop computers, desktop computers, and other mobile devices and portable media that store or process sensitive government information (including PII).

- iv. Verify that the encryption solutions in use have been validated under the Cryptographic Module Validation Program to confirm compliance with current FIPS 140 validation certificate from the NIST CMVP. The Contractor must provide a written copy of the validation documentation to the COR [CMS-provided delivery date].
 - v. Use the Key Management system on the HHS personal identification verification (PIV) card or establish and use a key recovery mechanism to ensure the ability for authorized personnel to encrypt/decrypt information and recover encryption keys <http://csrc.nist.gov/publications/>. Encryption keys must be provided to the COR upon request and at the conclusion of the contract.
1. **Contractor Non-Disclosure Agreement (NDA).** Each Contractor (and/or any subcontractor) employee having access to non-public government information under this contract must complete the CMS non-disclosure agreement. Contractors (and/or subcontractors) must submit a copy of each signed and witnessed NDA to the Contracting Officer (CO) and/or CO Representative (COR) prior to performing any work under this acquisition.
2. **Training Requirements**
 - a. **Mandatory Training for All Contractor Staff.** All Contractor (and/or any subcontractor) employees assigned to work on this contract must complete the applicable HHS/CMS Contractor Information Security Awareness, Privacy, and Records Management training (provided upon contract award) before performing any work under this contract. Thereafter, the employees must complete CMS Information Security Awareness, Privacy, and Records Management training at least **annually**, during the life of this contract. All provided training must be compliant with HHS training policies.
 - b. **Role-based Training.** All Contractor (and/or any subcontractor) employees with significant security responsibilities (as determined by the program manager) must complete role-based training **annually** commensurate with their role and responsibilities in accordance with HHS policy and the *HHS Role-Based Training (RBT) of Personnel with Significant Security Responsibilities Memorandum*.
 - c. **Training Records.** The Contractor (and/or any subcontractor) must maintain training records for all its employees working under this contract in accordance with HHS policy. A copy of the training records must be provided to the CO and/or COR within **30 days** after contract award and **annually** thereafter or upon request.
3. **Rules of Behavior**
 - a. The Contractor (and/or any subcontractor) must ensure that all employees performing on the contract comply with the *HHS Information Technology General Rules of Behavior*, and *HHS Rules of Behavior for Privileged Users*.
 - b. All Contractor employees performing on the contract must read and adhere to the Rules of Behavior before accessing Department data or other information, systems, and/or networks that store/process government information, initially at the beginning of the contract and at least **annually** thereafter, which may be done

as part of annual CMS Information Security Awareness Training. If the training is provided by the contractor, the signed ROB must be provided as a separate deliverable to the CO and/or COR per defined timelines above.

4. Incident Response

- a. The Contractor (and/or any subcontractor) must respond to all alerts/Indicators of Compromise (IOCs) provided by HHS Computer Security Incident Response Center (CSIRC)/ CMS IRT teams **within 24 hours**, whether the response is positive or negative.

FISMA defines an incident as "an occurrence that (1) actually or imminently jeopardizes, without lawful authority, the integrity, confidentiality, or availability of information or an information system; or (2) constitutes a violation or imminent threat of violation of law, security policies, security procedures, or acceptable use policies. In accordance with OMB M-17-12, *Preparing for and Responding to a Breach of Personally Identifiable Information (PII)*, an incident is "an occurrence that (1) actually or imminently jeopardizes, without lawful authority, the integrity, confidentiality, or availability of information or an information system; or (2) constitutes a violation or imminent threat of violation of law, security policies, security procedures, or acceptable use policies" and a privacy breach is "the loss of control, compromise, unauthorized disclosure, unauthorized acquisition, or any similar occurrence where (1) a person other than an authorized user accesses or potentially accesses personally identifiable information or (2) an authorized user accesses or potentially accesses personally identifiable information for an other than authorized purpose." For additional information on the HHS breach response process, please see the *HHS Policy and Plan for Preparing for and Responding to a Breach of Personally Identifiable Information (PII)*."

- b. In the event of a suspected or confirmed incident or breach, the Contractor (and/or any subcontractor) must:
 - i. Protect all sensitive information, including any PII created, stored, or transmitted in the performance of this contract, with encryption solution that is validated with current FIPS 140 validation certificate from the NIST CMVP.
 - ii. NOT notify affected individuals unless so instructed by the Contracting Officer or designated representative. If so instructed by the Contracting Officer or representative, the Contractor must send CMS approved notifications to affected individuals in accordance with the CMS Breach Analysis Team (BAT) instruction.
 - iii. Report all suspected and confirmed information security and privacy incidents and breaches to the CMS Incident Response Team (IRT) *via the CMS Help Desk (410) 786-2580 or CMS IT Service Desk (cms_it_service_desk@cms.hhs.gov)*, COR, CO, CMS SOP (or his or her designee), and other stakeholders, including breaches involving PII, in any medium or form, including paper, oral, or electronic, as soon as possible and without unreasonable delay, no later than **one (1) hour**, and consistent

with the applicable CMS and HHS policy and procedures, NIST standards and guidelines, as well as US-CERT notification guidelines. The types of information required in an incident report must include at a minimum: company and point of contact information, contact information, impact classifications/threat vector, and the type of information compromised. In addition, the Contractor must:

- Cooperate and exchange any information, as determined by the Agency, necessary to effectively manage or mitigate a suspected or confirmed breach;
 - Not include any sensitive information in the subject or body of any reporting e-mail; and
 - Encrypt sensitive information in attachments to email, media, etc.
- iv. Comply with OMB M-17-12, *Preparing for and Responding to a Breach of Personally Identifiable Information*, and HHS/CMS privacy breach response policies when handling PII breaches.
- v. Provide full access and cooperate on all activities as determined by the Government to ensure an effective incident response, including providing all requested images, log files, and event information to facilitate rapid resolution of sensitive information incidents. This may involve disconnecting the system processing, storing, or transmitting the sensitive information from the Internet or other networks or applying additional security controls. This may also involve physical access to contractor facilities during a breach/incident investigation.

5. Position Sensitivity Designations

All Contractor (and/or any subcontractor) employees must obtain a background investigation commensurate with their position sensitivity designation that complies with Parts 1400 and 731 of Title 5, Code of Federal Regulations (CFR). The following position sensitivity designation levels apply to this solicitation/contract:

Tier 1, Low-risk, Non-Sensitive

6. Homeland Security Presidential Directive (HSPD)-12

The Contractor (and/or any subcontractor) and its employees must comply with Homeland Security Presidential Directive (HSPD)-12, *Policy for a Common Identification Standard for Federal Employees and Contractors*; OMB M-05-24; OMB M-19-17; FIPS 201, *Personal Identity Verification (PIV) of Federal Employees and Contractors*; HHS HSPD-12 policy; and *Executive Order 13467, Part 1 §1.2*.

7. Roster

The Contractor (and/or any subcontractor) must submit a roster by name, position, e-mail address, phone number and responsibility, of all staff working under this acquisition where the Contractor will develop, have the ability to access, or host and/or maintain a government information system(s). The roster must be submitted to the COR and/or CO within a timeframe to be determined by CMS of the effective date of this contract. Any revisions to the roster as a result of staffing changes must be submitted within a

timeframe to be determined by CMS of the change. The COR will notify the Contractor of the appropriate level of investigation required for each staff member.

If the employee is filling a new position, the Contractor must provide a position description, and the Government will determine the appropriate suitability level.

8. Contract Initiation and Expiration

- a. **General Security Requirements.** The Contractor (and/or any subcontractor) must comply with information security and privacy requirements, Enterprise Performance Life Cycle (EPLC) processes, HHS Enterprise Architecture requirements to ensure information is appropriately protected from initiation to expiration of the contract. All information systems development or enhancement tasks supported by the contractor must follow the HHS EPLC framework and methodology or the CMS SDLC, as amended and in accordance with the HHS Contract Closeout Guide (2012).
- b. **System Documentation.** Contractors (and/or any subcontractors) must follow and adhere to HHS System Development Life Cycle requirements, at a minimum, for system development and provide system documentation at designated intervals (specifically, at the expiration of the contract) within the EPLC and CMS SDLC that require artifact review and approval.
- c. **Sanitization of Government Files and Information.** As part of contract closeout and at expiration of the contract, the Contractor (and/or any subcontractor) must provide all required documentation in accordance with the CMS SDLC, as Amended to the CO and/or COR to certify that, at the government's direction, all electronic and paper records are appropriately disposed of and all devices and media are sanitized in accordance with NIST SP 800-88, *Guidelines for Media Sanitization*.
- d. **Notification.** The Contractor (and/or any subcontractor) must notify the CO and/or COR and system ISSO within as soon as possible or as determined by CMS before an employee stops working under this contract.
- e. **Contractor Responsibilities upon Physical Completion of the Contract.** The contractor (and/or any subcontractors) must return all government information and IT resources (i.e., government information in non-government-owned systems, media, and backup systems) acquired during the term of this contract to the CO and/or COR. Additionally, the Contractor must provide a certification that all government information has been properly sanitized and purged from Contractor-owned systems, including backup systems and media used during contract performance, in accordance with HHS and/or CMS policies.
- f. The Contractor (and/or any subcontractor) must perform and document the actions identified the COR when an employee terminates work under this contract within 30 calendar days of the employee's exit from the contract. All documentation must be available to the CO and/or COR upon request.

9. Records Management and Retention

- a. The Contractor (and/or any subcontractor) must maintain all information in accordance with Executive Order 13556 -- Controlled Unclassified Information,

National Archives and Records Administration (NARA) records retention policies and schedules and *HHS Policy for Records Management* and CMS policies and must not dispose of any records unless authorized by HHS/CMS.

- b. In the event that a contractor (and/or any subcontractor) accidentally disposes of or destroys a record without proper authorization, he/she must document and report the incident in accordance with HHS/CMS policies.

10. High Value Asset (HVA)

If a system is identified as HVA, the contractor must comply with the HHS Policy for the High Value Asset (HVA) Program and the DHS HVA Control Overlay in addition to the above requirements.

6.2 Privacy Act

It has been determined that this contract is subject to the Privacy Act of 1974, because this contract provides for the design, development, or operation of a system of records about individuals from which records are retrieved by name or other identifying particular.

The System of Records Notice (SORN) that is applicable to this contract is: Notice No. 09-70-0560

The design, development, or operation work the Contractor is to perform is: to maintain, update and enhance the CCIIO Enrollment Resolution and Reconciliation System (CERRS), System Exchange Enrollment Data (SEED), CERRS Marketplace Consumer Record (MCR), Health Insurance Casework System (HICS), and the Enrollment Data Sandbox. In addition, development and operation work is required for managing CERRS and HICS in the CMS Microsoft Azure Government (MAG). This work includes code escalation, monitoring, testing and evaluation of the implementation of changes and operations

The disposition to be made of the Privacy Act records upon completion of contract performance is: Cutoff end of CY. Destroy 10 years after record is closed, but longer retention is authorized. Records stored and managed in Microsoft Azure Government (MAG) cloud.

6.3 HHS FedRAMP Privacy and Security Requirements

The Contractor (and/or any subcontractor) must be responsible for the following privacy and security requirements:

- a. **FedRAMP Compliant ATO.** Comply with FedRAMP Assessment and Authorization (A&A) requirements and ensure the information system/service under this contract has a valid FedRAMP compliant (approved) authority to operate (ATO) in accordance with Federal Information Processing Standard (FIPS) Publication 199 defined security categorization. If a FedRAMP compliant ATO has not been granted, the Contractor must submit a plan to obtain a FedRAMP compliant ATO by a timeline determined by CMS.
 - i. Implement applicable FedRAMP baseline controls commensurate with the agency-defined security categorization and the applicable FedRAMP

security control baseline (www.FedRAMP.gov). The *HHS Information Security and Privacy Policy (IS2P)*, *HHS Cloud Computing and Federal Risk and Authorization Management Program (FedRAMP) Guidance*, and the CMS Information System Security and Privacy Policy (IS2P2) further define the baseline policies as well as roles and responsibilities. The Contractor must also implement a set of additional controls identified by the agency when applicable.

- ii. A security control assessment must be conducted by a FedRAMP third-party assessment organization (3PAO) for the initial ATO and **annually** thereafter or whenever there is a significant change to the system's security posture in accordance with the FedRAMP Continuous Monitoring Plan.
 - b. **Data Jurisdiction.** The contractor must store all information within the security authorization boundary, data at rest or data backup, within the Continental United States (CONUS) if so required. Refer to the following clause - Contractor Work Performed Outside of the United States and its Territories (April 2016) in the Solicitation/Contract.
 - c. **Service Level Agreements.** The Contractor must understand the terms of the service agreements that define the legal relationships between cloud customers and cloud providers and work with CMS to develop and maintain an SLA.
 - d. **Interconnection Agreements/Memorandum of Agreements.** The Contractor must establish and maintain Interconnection Agreements and or Memorandum of Agreements/Understanding in accordance with HHS/CMS policies.
2. Protection of Information in a Cloud Environment
- a. If contractor (and/or any subcontractor) personnel must remove any information from the primary work area, they must protect it to the same extent they would the proprietary data and/or company trade secrets and in accordance with HHS/CMS policies <https://www.hhs.gov/web/governance/digital-strategy/it-policy-archive/index.html>.
 - b. HHS/CMS will retain unrestricted rights to federal data handled under this contract. Specifically, HHS/CMS retains ownership of any user created/loaded data and applications collected, maintained, used, or operated on behalf of HHS/CMS and hosted on contractor's infrastructure, as well as maintains the right to request full copies of these at any time. If requested, data must be available to HHS/CMS within **one (1) business day** from request date or within the timeframe specified otherwise. In addition, the data must be provided at no additional cost to HHS/CMS.
 - c. The Contractor (and/or any subcontractor) must ensure that the facilities that house the network infrastructure are physically and logically secure in accordance with FedRAMP requirements and HHS/CMS policies.
 - d. The contractor must support a system of records in accordance with NARA-approved records schedule(s) and protection requirements for federal agencies to manage their electronic records in accordance with 36 CFR § 1236.20 & 1236.22 (ref. a), including but not limited to the following:
 - i. Maintenance of links between records and metadata, and

- ii. Categorization of records to manage retention and disposal, either through transfer of permanent records to NARA or deletion of temporary records in accordance with NARA-approved retention schedules.
- e. The disposition of all HHS/CMS data must be at the written direction of HHS/CMS. This may include documents returned to HHS/CMS control; destroyed; or held as specified until otherwise directed. Items returned to the Government must be hand carried or sent by certified mail to the COR.
 - i. If the system involves the design, development, or operation of a system of records on individuals, the Contractor must comply with the Privacy Act requirements.

3. Assessment and Authorization (A&A) Process

- a. The Contractor (and/or any subcontractor) must comply with HHS/CMS and FedRAMP requirements as mandated by federal laws, regulations, and HHS/CMS policies, including making available any documentation, physical access, and logical access needed to support the A&A requirement. The level of effort for the A&A is based on the system's FIPS 199 security categorization and HHS/CMS security policies. The contractor must obtain authorization prior to deployment or service implementation.
 - i. In addition to the FedRAMP compliant ATO, the contractor must complete and maintain an agency A&A package to obtain agency ATO prior to system deployment/service implementation within CFACTS in a timeline to be determined by CMS. The agency ATO must be approved by the CMS authorizing official (AO) prior to implementation of system and/or service being acquired.
 - ii. CSP systems categorized as Federal Information Processing Standards (FIPS) 199 high must leverage a FedRAMP accredited third-party assessment organization (3PAO); moderate impact CSP systems must make a best effort to use a FedRAMP accredited 3PAO but should not use self-assessment. CSP systems categorized as FIPS 199 low impact may leverage a non-accredited, independent assessor.
 - iii. For all acquired cloud services, the A&A package must contain the following documentation:
 - Security Assessment Plan (SAP)/Security Assessment Report (SAR)
 - System Security Plan (SSP)
 - Plan of Action and Milestones
 - Contingency Plan and Contingency Plan Test
 - E-Authentication Questionnaire].

Following the initial ATO, the Contractor must review and maintain the ATO in accordance with HHS/CMS policies using CMS templates and defined timelines.
- b. HHS/CMS reserves the right to perform penetration testing (pen testing) on all systems operated on behalf of agency. If HHS/CMS exercises this right, the Contractor (and/or any subcontractor) must allow HHS/CMS employees (and/or designated third parties) to conduct Security Assessment activities to include

control reviews in accordance with HHS/CMS requirements. Review activities include, but are not limited to, scanning operating systems, web applications, wireless scanning; network device scanning to include routers, switches, and firewall, and IDS/IPS; databases and other applicable systems, including general support structure, that support the processing, transportation, storage, or security of Government information for vulnerabilities.

- c. The Contractor must identify any gaps between required FedRAMP Security Control Baseline/Continuous Monitoring controls and the contractor's implementation status as documented in the Security Assessment Report and related Continuous Monitoring artifacts. In addition, the contractor must document and track all gaps for mitigation in a Plan of Action and Milestones (POA&M) document. Depending on the severity of the risks, HHS/CMS may require remediation at the contractor's expense, before HHS/CMS issues an ATO.
 - d. The Contractor (and/or any subcontractor) must mitigate security risks for which they are responsible, including those identified during A&A and continuous monitoring activities. All vulnerabilities and findings must be remediated, in accordance with timelines specified in the HHS POA&M Standard, from discovery: (1) critical vulnerabilities no later than *fifteen (15) days* and (2) high within *thirty (30) days* (3) medium within *ninety (90) days* and (4) low vulnerabilities no later than *three hundred and sixty (360)*. In the event a vulnerability or other risk finding cannot be mitigated within the prescribed timelines above, they must be added to the designated POA&M and mitigated within the timelines assigned to the POA&M within CFACTS. HHS/CMS will determine the risk rating of vulnerabilities using FedRAMP baselines.
 - e. **Revocation of a Cloud Service.** HHS/CMS have the right to take action in response to the CSP's lack of compliance and/or increased level of risk. In the event the CSP fails to meet HHS/CMS and FedRAMP security and privacy requirements and/or there is an incident involving sensitive information, HHS and/or CMS may suspend or revoke an existing agency ATO (either in part or in whole) and/or cease operations. If an ATO is suspended or revoked in accordance with this provision, the CO and/or COR may direct the CSP to take additional security measures to secure sensitive information. These measures may include restricting access to sensitive information on the Contractor information system under this contract. Restricting access may include disconnecting the system processing, storing, or transmitting the sensitive information from the Internet or other networks or applying additional security controls.
4. Reporting and Continuous Monitoring
- a. Following the initial ATOs, the Contractor (and/or any subcontractor) must perform the minimum ongoing continuous monitoring activities specified below, submit required deliverables by the specified due dates, and meet with the system/service owner and other relevant stakeholders to discuss the ongoing continuous monitoring activities, findings, and other relevant matters. The CSP will work with the agency to schedule ongoing continuous monitoring activities. Meetings shall be held at least monthly, or as needed.

- b. At a minimum, the Contractor must provide the following artifacts/deliverables on a monthly basis via the CCIC requirements which can be provided by contacting EVM-CMP@cms.hhs.gov. Deliverables are identified as:
 - i. Operating system, database, Web application, and network vulnerability scan results;
 - ii. Updated POA&Ms;
 - iii. Any updated authorization package documentation as required by the annual attestation/assessment/review or as requested by the CMS System Owner or AO; and
 - iv. Any configuration changes to the system and/or system components or CSP's cloud environment, that may impact HHS/CMS security posture. Changes to the configuration of the system, its components, or environment that may impact the security posture of the system under this contract must be approved by the agency.
5. Configuration Baseline
- a. The contractor must certify that applications are fully functional and operate correctly as intended on systems using *HHS Minimum Security Configurations Standards Guidance*. The standard installation, operation, maintenance, updates, and/or patching of software must not alter the configuration settings from the approved HHS/CMS configuration baseline.
 - b. The contractor must use Security Content Automation Protocol (SCAP) validated tools with configuration baseline scanner capability to certify their products operate correctly with HHS/CMS and NIST defined configurations and do not alter these settings.
6. Incident Reporting
- a. The Contractor (and/or any subcontractor) must provide an Incident and Breach Response Plan (IRP) in accordance with HHS/CMS/, OMB, and US-CERT requirements and obtain approval from CMS. In addition, the Contractor must follow the incident response and US-CERT reporting guidance contained in the FedRAMP Incident Communications.
 - b. The Contractor (and/or any subcontractor) must implement a program of inspection to safeguard against threats and hazards to the security, confidentiality, integrity, and availability of federal data, afford HHS/CMS access to its facilities, installations, technical capabilities, operations, documentation, records, and databases within 72 hours of notification. The program of inspection must include, but is not limited to:
 - i. Conduct authenticated and unauthenticated operating system/network/database/Web application vulnerability scans. Automated scans can be performed by HHS/CMS personnel, or agents acting on behalf of HHS/CMS, using agency-operated equipment and/or specified tools. The Contractor may choose to run its own automated scans or audits, provided the scanning tools and configuration settings are compliant with NIST Security Content Automation Protocol (SCAP)

standards and have been approved by the agency. The agency may request the Contractor's scanning results and, at the agency discretion, accept those in lieu of agency performed vulnerability scans.

- ii. In the event an incident involving sensitive information occurs, cooperate on all required activities determined by the agency to ensure an effective incident or breach response and provide all requested images, log files, and event information to facilitate rapid resolution of sensitive information incidents. In addition, the Contractor must follow the agency reporting procedures and document the steps it takes to contain and eradicate the incident, recover from the incident, and provide a post-incident report that includes at a minimum the following:
 - Company and point of contact name;
 - Contract information;
 - Impact classifications/threat vector;
 - Type of information compromised;
 - A summary of lessons learned; and
 - Explanation of the mitigation steps of exploited vulnerabilities to prevent similar incidents in the future.

7. Media Transport

- a. The Contractor and its employees must be accountable and document all activities associated with the transport of government information, devices, and media transported outside controlled areas and/or facilities. These include information stored on digital and non-digital media (e.g., CD-ROM, tapes, etc.), mobile/portable devices (e.g., USB flash drives, external hard drives, and SD cards).
- b. All information, devices and media must be encrypted with HHS/CMS-approved encryption mechanisms to protect the confidentiality, integrity, and availability of all government information transported outside of controlled facilities.

8. Boundary Protection: Trusted Internet Connections (TIC)

- a. The contractor must ensure that government information, other than unrestricted information, being transmitted from federal government entities to external entities using cloud services is inspected by Trusted Internet Connection (TIC) processes that are in compliance with the requirements of the Office of Management and Budget (OMB) Memorandum (M) 19-26: Update to the TIC Initiative, TIC 3.0.
- b. The contractor must route all external connections through a TIC.
- c. **Non-Repudiation.** The contractor must provide a system that implements encryption with current FIPS 140 validation certificate from the NIST CMVP that provides for origin authentication, data integrity, and signer non-repudiation.

Appendix A: Applicable References / Documents

The following are provided as supporting references to this SOW.

General Regulations and Guidance

- Patient Protection and Affordable Care Act; Establishment of Exchanges and Qualified Health Plans [CMS-9989-F], published on March 27, 2012, and found at:
<http://www.gpo.gov/fdsys/pkg/FR-2012-03-27/pdf/2012-6125.pdf>
- Patient Protection and Affordable Care Act; Exchange and Insurance Market Standards for 2015 and Beyond [CMS-9949-F2] published on May 27, 2014, and found at:
<http://www.gpo.gov/fdsys/pkg/FR-2014-05-27/pdf/2014-11657.pdf>
- Enrollment Manual, published on August 19, 2024, and found at:
https://regtap.cms.gov/reg_library_openfile.php?id=5507&type=1
- Form 1095-A, Health Insurance Exchange Statement, and found at:
<https://www.irs.gov/pub/irs-pdf/f1095a.pdf>
- Instructions for Form 8962, and found at:
<http://www.irs.gov/pub/irs-dft/i8962--dft.pdf>

Appendix B: Sample Notice List

Notices
Death Periodic Data Matching notices - English
Death Periodic Data Matching notices - Spanish
ER&R Third Failed Attempt Letter - English
ER&R Third Failed Attempt Letter - Spanish
Form 1095-A Corrected with English Cover Letter
Form 1095-A Corrected with Spanish Cover Letter
Form 1095-A denial notice 1A - Initial denial letter in English
Form 1095-A denial notice 1A - Initial denial letter in Spanish
Form 1095-A denial notice 1B - Initial denial with phone call attempt in English
Form 1095-A denial notice 1B - Initial denial with phone call attempt in Spanish
Form 1095-A denial notice 1C - Second denial with escalation after phone call in English
Form 1095-A denial notice 1C - Second denial with escalation after phone call in Spanish
Form 1095-A denial notice 2 - Final denial with mailing information for Statements of Disagreement in English
Form 1095-A denial notice 2 - Final denial with mailing information for Statements of Disagreement in Spanish
Form 1095-A Initial with English Cover Letter
Form 1095-A Initial with Spanish Cover Letter
Form 1095-A Unaffiliated Issuer Enrollment (UIE)
Form 1095-A Voided with English Cover Letter
Form 1095-A Voided with Spanish Cover Letter
Proof of Non-Coverage
SEP Resolved Approved - English
SEP Resolved Approved - Spanish
SEP Resolved Approved Retro Denied - English
SEP Resolved Approved Retro Denied- Spanish
SEP Resolved Denied - English
SEP Resolved Denied - Spanish
SEP Resolved Retro Approved - English
SEP Resolved Retro Approved - Spanish
SEP Resolved Retro Denied - English
SEP Resolved Retro Denied - Spanish
SEP Resolved Retro Term Denied - English
SEP Resolved Retro Term Denied - Spanish
SEP Worked Unable to Contact - English
SEP Worked Unable to Contact - Spanish

Appendix C: Sample List of SOPs

Document Name
1095-A Consumer Disputes Standard Operating Procedures (SOP)
Complex Case Help Center (CCHC) Casework Standard Operating Procedure (SOP)
ER&R Batch Update Utility Standard Operating Procedure (SOP)
ER&R Business Rule Development Standard Operating Procedure (SOP)
ER&R Support Center Standard Operating Procedure (SOP)
Issuer Enrollment & Payment Dispute Standard Operating Procedure (SOP)
Issuer Outreach Standard Operating Procedure (SOP)
Unmatched I Records (UIR) Standard Operating Procedure (SOP)

Appendix D: Sample List of User Guides and Job Aids

Document Name
CCHC CERRS Field Guide
CCHC Consumer Communications Guidance
CCHC Enhanced Access User Guide Complex Case Help Center (CCHC)
CCHC Root Cause Definitions
CCIO Payment Dispute Flow
CERRS 1095-A Field Guide
Closeout Process User Guide
Coaching Batch Update Utility (BUU) Errors
Common DMI Types
Complex Case Help Center (CCHC) Casework User Guide
Consumer Disputes Case Notes Guidance
Consumer Outreach Call Scripts
DMI Overview
Enrollment Dispute Roadmap
ER&R 1095-A Privacy Authorization Guide
ER&R/1095-A Acronyms and Definitions Glossary
ER&R/1095-A General Casework and Call-Related Principles
ER&R/1095-A Issue/Outage Checklist
ER&R/1095-A Privacy Authorization Guide
ER&R/1095-A Quality Checklist
ER&R/1095-A Style Guide
HICS Direct Disputes Job Aide
Manual CCHC Case Creation Process
MCR Search Instruction Manual
Language Line Job Aid
Language Line Job Aid Complex Case Help Center (CCHC)
Quick Reference Guide - Acceptable DMI Documentation
Reporting User Guide
Telephony System User Guide

Appendix E: Contractor Non-Disclosure Agreement

NONDISCLOSURE AGREEMENT FOR CONTRACTOR EMPLOYEES

Subject: Contract No. _____

Reference: Contract Clause G-4, Dissemination, Publication and Distribution of Information

I, _____ (print or type name), as an employee of _____ (insert name of company), a contractor acting under contract to the Centers for Medicare & Medicaid Services (CMS), pursuant to the subject contract, agree, as a condition of being granted access to information related to this contract, not to disclose to any individual business entity or anyone within _____ (insert name of company) or outside of the company who has not signed a nondisclosure agreement for the purposes of performing this contract: any sensitive, proprietary or source selection information contained or accessible through the subject contract. Proprietary information/data will be handled in accordance with Government regulations.

I understand that information/data I may be aware of, or possess, as a result of my assignment under this contract may be considered sensitive or proprietary by the Government. The Contractor's responsibility for proper use and protection from unauthorized disclosure of sensitive, proprietary and source selection information is described in Federal Acquisition Regulation (FAR) section [3.104-4](#).

Pursuant to [FAR 3.104-4](#), I agree not to appropriate such information for my own use or to release or discuss such information for my own use or to release it to or discuss it with third parties unless specifically authorized in writing to do so, as provided above. Should my work entail design, development or operation of a Federal system of records violations may result in the imposition of criminal penalties, See, [HHSAR 352.224-70](#).

THIS CERTIFICATION CONCERNS A MATTER WITHIN THE JURISDICTION OF AN AGENCY OF THE UNITED STATES AND THE MAKING OF A FALSE, FICTITIOUS, OR FRAUDULENT CERTIFICATION MAY RENDER THE MAKER SUBJECT TO PROSECUTION UNDER TITLE 18, UNITED STATES CODE, SECTION 1001.

Signature of Contractor Employee _____

Date _____

Print Name _____

Contractor _____

Employee Telephone _____

Appendix F: List of Security and Privacy Deliverables

Document Section	Deliverable Title/Description	Due Date	Applicable (y/n)
2 – Roster	Roster	<i>To be determined by CMS after contract award</i>	Y
2 – Contractor Employee Non-Disclosure Agreement (NDA)	Contractor Employee Non-Disclosure Agreement (NDA)	Prior to performing any work on behalf of HHS/ <u>CMS</u>	Y
2 – Privacy Threshold Analysis (PTA)/ Privacy Impact Assessment (PIA)	Assist in the completion of a PTA/PIA form	<i>To be determined by CMS after contract award</i>	Y
2 – Training Records	Copy of training records for all mandatory training	In conjunction with contract award and annually thereafter or upon request	Y
2 – Rules of Behavior	Signed ROB for all employees	Initiation of contract and at least annually thereafter	Y
2 – Incident Response	Incident Report (as incidents or breaches occur)	As soon as possible and without reasonable delay and no later than 1 hour of discovery	Y
2 – Incident Response	Incident and Breach Response Plan	Upon request from <u>CMS</u>	Y
2 – Personnel Security Responsibilities	List of Personnel with defined roles and responsibilities	<i>To be determined by CMS after contract award</i>	Y
2 – Personnel Security Responsibilities	Off-boarding documentation, equipment and badge when leaving contract	<i>To be determined by CMS after contract award</i>	Y
2 – Background Investigation	Onboarding documentation when beginning contract.	Prior to performing any work on behalf of <u>CMS</u>	Y
2 - Certification of Sanitization of Government and Government Activity-Related Files, Information, and Devices.	Form or deliverables required by <u>CMS</u> .	At contract expiration.	Y

Document Section	Deliverable Title/Description	Due Date	Applicable (y/n)
2 – Contract Initiation and Expiration	If the procurement involves a system or cloud service, additional documentation will be required, such as Disposition/Decommission Plan	At contract expiration.	Y
4 – Security Assessment and Authorization (SA&A)	SA&A Package • SSP • SAR • POA&M • Authorization Letter • CP and CPT Report • E-Auth (if applicable) • PTA/PIA (if applicable) • Interconnection/Data Use Agreements (if applicable) • Authorization Letter • Configuration Management Plan (if applicable) • Configuration Baseline • Other <u>CMS</u> -specific documents	<u>To be determined by CMS after contract award</u>	Y
5 – Protection of Information in a Cloud Environment	Contract expiration	<u>To be determined by CMS after contract award</u>	Y
5 – SA&A Process for Cloud Services	SA&A Package • SSP • SAR • POA&M • CMP • CP and CPT Report • E-Auth (if applicable) • PTA/PIA (if applicable) • Penetration Test Results • Interconnection/Data Use/Agreements (if applicable) • Service Level Agreement • Authorization Letter • Configuration Management Plan (if applicable)	<u>To be determined by CMS after contract award</u>	Y

Document Section	Deliverable Title/Description	Due Date	Applicable (y/n)
	- Configuration Baseline - Other <u>CMS</u> -specific documents		
5 – Reporting and Continuous Monitoring	POA&M updates; Revised security documentation/Agreements	<u>Quarterly/as requested by CMS</u>	Y
5 – Security Alerts, Advisories, and Directives	List of personnel with designated roles and responsibilities	<u>To be determined by CMS after contract award</u>	Y
5 – Incident Reporting	- Incident reports (as needed) - Incident Response Plan	<u>As needed in coordination with CMS Incident Management Team</u>	Y
6 – Other IT Procurements (Non-Commercial and Open-Source Computer Software Procurements)	Computer software, including the source code.	Prior to performing any work on behalf of <u>CMS</u>	Y

References

1. Federal Laws, Directives and Policies

United States Code (U.S.C.) of Federal Regulation (CFR), as amended (<http://www.ecfr.gov/cgi-bin/ECFR?page=browse>).

Federal Acquisition Regulation (FAR), as amended (<https://www.acquisition.gov/?q=browsefar>).

Federal Information Security Management Act (FISMA) of 2002 (Pub. L. No. 107-347, Title III) (<http://csrc.nist.gov/>).

Federal Information Security Modernization Act (FISMA) of 2014 (44 U.S.C. 101), as amended (<https://www.congress.gov/>).

Federal Information Technology Acquisition Reform Act (FITARA) H.R. 1232, as amended (<https://www.congress.gov/>).

Executive Order 13556 – *Controlled Unclassified Information (CUI)*, (implemented at 3 CFR, Part 2002), as amended (<https://obamawhitehouse.archives.gov/>).

Homeland Security Presidential Directive 12 (HSPD-12), *Policy for a Common Identification Standard for Federal Employees and Contractors*, as amended (<https://www.dhs.gov/>).

FedRAMP Standard Contract Clauses, as amended (<http://www.fedramp.gov/>).

FedRAMP Control-Specific Contract Clauses, as amended (<http://www.fedramp.gov/>).

The Health Insurance Portability and Accountability Act (HIPAA), as amended (<https://www.hhs.gov/hipaa/for-professionals/index.html>).

The Health Information Technology for Economic and Clinical Health Act (HITECH), as amended (<https://www.healthit.gov/>).

National Archives and Records Administration (NARA) Bulletin 20013-02 (2013), as amended (www.archives.gov).

National Industrial Security Program Operating Manual (NISPOM), as amended (<https://intranet.hhs.gov/security/index.html>).

Office of Personnel Management (OPM) Position Sensitivity Designation Automated Tool, as amended (<https://www.opm.gov/investigations/>).

The Privacy Act of 1974, as amended (<https://www.justice.gov/opcl/privacy-act-1974>).

Public Law 96-511 Paperwork Reduction Act of 1980, as amended (<https://www.gpo.gov/>).

United States Government Configuration Baseline (USGCB), as amended (<http://usgcb.nist.gov/>).

OMB Policy and Memoranda (<https://obamawhitehouse.archives.gov/omb/>)

OMB Circular A-130, *Managing Information as a Strategic Resource*, as amended.

OMB Encryption Guidance, as amended.

OMB M-17-12, *Preparing for and Responding to a Breach of PII*.

OMB Memorandum 07-18, *Ensuring New Procurements Include Common Security Configurations*, as amended.

OMB M-07-16, *Safeguarding Against and Responding to the Breach of Personally Identifiable Information*,

OMB M-06-19, *Safeguarding Against and Responding to the Breach of Personally Identifiable Information*.

OMB M-06-15, *Safeguarding Personally Identifiable Information*.

OMB M-04-16, *Software Acquisition*.

OMB M-04-04, *E-Authentication Guidance for Federal Agencies*.

OMB M-03-22, *Guidance for Implementing the Privacy Provisions of the E-Government Act of 2002*.

2. HHS Policy

HHS Cloud Computing and Federal Risk and Authorization Management Program [FedRAMP] Guidance, as amended (<http://intranet.hhs.gov/it/cybersecurity/policies/>).

HHS Contract Closeout Guide, as amended, (<http://intranet.hhs.gov/abouthhs/contracts-grants-support/>).

HHS Enterprise Performance Life Cycle [EPLC], as amended (<http://www.hhs.gov/ocio/>).

HHS Minimum Security Configuration Standards for Departmental Operating Systems and Applications, as amended (http://intranet.hhs.gov/it/cybersecurity/docs/policies_guides/).

HHS Role-Based Training (RBT) of Personnel with Significant Security Responsibilities Memorandum, as amended (<http://intranet.hhs.gov/it/cybersecurity/policies/index.html>).

HHS-OCIO *Policy for Information Systems Security and Privacy Policy*, as amended (<http://intranet.hhs.gov/it/ocio/>).

HHS *Guidance for Purchasing Noncommercial Computer Software and “Open Source” Licenses*, as amended (<http://www.hhs.gov/ocio/policy/>).

HHS Standard for the *Definition of Sensitive Information*, as amended (http://intranet.hhs.gov/it/cybersecurity/docs/policies_guides/).

HHS Standard for Encryption of Computing Devices and Information, as amended (<https://intranet.hhs.gov/it/cybersecurity/policies/index.html>).

HHS *Standard for Plan of Action and Milestones*, as amended (<http://intranet.hhs.gov/it/cybersecurity/policies/>).

Health and Human Services Acquisition Regulation (HHSAR), as amended (<https://www.gpo.gov/fdsys/pkg/CFR-2015-title48-vol4/pdf/CFR-2015-title48-vol4-chap3.pdf>).

HHS Personnel Security & Suitability Handbook, as amended (<http://intranet.hhs.gov/security/ossi/documents/>).

HHS Privacy Incident Response Team [PIRT] Standard Operating Procedures [SOP], as amended (<http://intranet.hhs.gov/it/cybersecurity/docs/>).

Rules of Behavior for use of HHS Information Resources, as amended (<http://www.hhs.gov/ocio/policy/>).

HHS Security and Privacy Language for Information and Information Technology Procurements v2, as amended (<https://intranet.hhs.gov/it/strategy-policy-governance/policies-standards-guides/standards/index.html>)

3. NIST Guidance (<http://csrc.nist.gov/publications/>)

NIST IR 7511 Revision 4, *Security Content Automation Protocol (SCAP) Version 1.2 Validation Program Test Requirements*, as amended.

NIST SP 800-171 Rev 1, *Protecting Controlled Unclassified Information in Nonfederal Information Systems and Organizations*, as amended.

NIST SP 800-145, *The NIST Definition of Cloud Computing*, as amended.

NIST SP 800-137, *Information Security Continuous Monitoring (ISCM) for Federal Information Systems and Organizations*, as amended.

NIST SP 800-124, Rev. 1, *Guidelines for Managing the Security of Mobile Devices in the Enterprise*, as amended.

NIST SP 800-122, *Guide to Protecting the Confidentiality of Personally Identifiable Information*, as amended.

NIST SP 800-88 Rev 1, *Guidelines for Media Sanitization*, as amended.

NIST SP 800-64 Revision 2, *Security Considerations in the Information System Development Lifecycle*, as amended.

NIST SP 800-63-3, *Digital Identity Guidelines: Authentication and Lifecycle Management*.

NIST SP 800-61 Rev 2, *Computer Security Incident Handling Guide*, as amended.

NIST SP 800-60 Revision 1, *Guide for Mapping Types of Information and Information Systems to Security Categories: (2 Volumes) – Volume 1: Guide, Volume 2: Appendices*, as amended.

NIST SP 800-53 Revision 4, *Security and Privacy Controls for Federal Information Systems and Organizations*, as amended.

NIST SP 800-53A Revision 1, *Guide for Assessing the Security Controls in Federal Information Systems and Organizations, Building Effective Security Assessment Plans*, as amended.

NIST SP 800-37 Revision 1, *Guide for Applying the Risk Management Framework to Federal Information Systems: A Security Life Cycle Approach*, as amended.

NIST SP 800-34 Revision 1, *Contingency Planning Guide for Information Technology Systems*, as amended.

NIST SP 800-30 Revision 1, *Guide for Conducting Risk Assessments*, as amended.

NIST SP 800-18 Revision 1, *Guide for Developing Security Plans for Federal Information Systems*, as amended.

FIPS PUB 200, *Minimum Security Requirements for Federal Information and Information Systems*, as amended.

FIPS PUB 201-2, *Personal Identity Verification (PIV) of Federal Employees and Contractors*, as amended.

FIPS PUB 199, *Standards for Security Categorization of Federal Information and Information Systems*, as amended.

FIPS 140-2, *Security Requirements for Cryptographic Modules*, as amended.

4. CMS Guidance

CMS Information Systems Security and Privacy Policy (IS2P2), as amended (<https://www.cms.gov/Research-Statistics-Data-and-Systems/CMS-Information-Technology/InformationSecurity/Info-Security-Library-Items/CMS-Information-Systems-Security-and-Privacy-Policy-IS2P2.html?DLPage=2&DLEntries=10&DLSort=0&DLSortDir=ascending>)

CMS Acceptable Risk Safeguards (ARS), as amended (<https://www.cms.gov/Research-Statistics-Data-and-Systems/CMS-Information-Technology/InformationSecurity/Info-Security-Library-Items/ARS-31-Publication.html?DLPage=1&DLEntries=10&DLSort=0&DLSortDir=ascending>)

CMS Target Life Cycle (TLC), as amended (<https://www.cms.gov/Research-Statistics-Data-and-Systems/CMS-Information-Technology/TLC/index>)

Acronyms

Acronym	Description
ASA	Assistant Secretary for Administration
ASFR	Office of the Assistant Secretary for Financial Resources
BI	Background Investigation
CIO	Chief Information Officer
CISO	Chief Information Security Officer
COR	Contracting Officers Representative
CSIRC	Computer Security Incident Response Center
CSIRT	Computer Security Incident Response Team
CSP	Cloud Service Provider
DA	Division of Acquisition
DSRS	Document Storage Retrieval System
E-Auth	Electronic Authentication
FAR	Federal Acquisition Regulations
FIPS	Federal Information Processing Standards
FISMA	Federal Information Security Modernization Act of 2014
FY	Fiscal Year
GB	GigaByte
HHS	Department of Health and Human Services
HHSAR	Health and Human Services Acquisition Regulations
HIPAA	Health Insurance Portability and Accountability Act
HSPD	Homeland Security Presidential Directive
IS2P	Information Systems Security and Privacy Policy
ISSO	Information Systems Security Officer
IT	Information Technology
M	Memorandum
MBI	Minimum Background Investigation
NARA	National Archives and Records Administration
NDA	Non-Disclosure Agreement
NIST	National Institute of Standards and Technology
OCIO	Office of the Chief Information Officer
OGAPA	Office of Grants and Acquisition Policy and Accountability
OMB	Office of Management and Budget
OpDiv	Operating Division
OSSI	Office of Security and Strategic Information
PII	Personally Identifiable Information
PIV	Personal Identification Verification
PL	Public Law
PM	Program Manager
RBT	Role Based Training
RFP	Request for Proposal
RoB	Rules of Behavior
SCAP	Security Content Automation Protocol
SORN	System of Record Notice
SOW	Statement of Work
SOP	Senior Official for Privacy
SP	Special Publication

Acronym	Description
SSP	System Security Plan
STAFFDIV	Staff Division
TBD	To be determined
USGCB	United States Government Configuration Baseline
USC	United States Code